Started on	Sunday, 24 July 2022, 3:35 PM
State	Finished
Completed on	Sunday, 24 July 2022, 3:36 PM
Time taken	8 secs
Marks	0.00/146.00
Grade	**0.00** out of 100.00

Question **1**

Not answered

Marked out of 1.00

Flag question

The diagnosis of induced delusional disorder is most commonly found in which of the following groups?

Select one:

- Family blood relations
- Couple relationships
- Children and adolescence
- Groups of men rather than women
- Groups of women rather than men

Explanation:

Most cases of induced delusional disorder occur within the nuclear family, usually between married or common-law couples (52%) or sisters (24%).

Ref: Cipriani G et al. A contagious disorder: Folie à deux and dementia. American Journal of Alzheimer's Disease & Other Dementias®. 2018 Nov;33(7):415-22.

The correct answer is: Couple relationships

Question **2**

Not answered

Marked out of 1.00

Flag question

The multi-axial version of ICD, prior to ICD-11, consisted of how many axes?

Select one:

- Five
- Four
- Three
- Ten
- One

Explanation:

The multi-axial version of ICD-10 used three axes: Axis 1 - the mental disorder (also personality disorder and mental handicap); Axis 2 - the degree of disability; and Axis 3 - current psychosocial

problems.

The correct answer is: Three

Question **3**

Not answered

Marked out of 1.00

Flag question

Previous systems of classification such as ICD-10 and DSM-IV described various descriptive subtypes of schizophrenia. Which one of the following of those historical subtypes was associated with the worst prognosis?

Select one:

- Residual schizophrenia
- Catatonic schizophrenia
- Post schizophrenic depression
- Hebephrenic schizophrenia
- Paranoid schizophrenia

Explanation:

The onset of hebephrenic schizophrenia is insidious, usually in the early second decade. The course is often progressively downhill. There is a rapid development of negative symptoms and disorganised behaviour. Severe deterioration without remission occurs over time. A recovery from the first episode was said to be rare in typical cases of the hebephrenic syndrome. But the validity of these subtypes were increasingly questioned, leading to their drop from modern classifications such as DSM-5 and ICD-11.

The correct answer is: Hebephrenic schizophrenia

Question **4**

Not answered

Marked out of 1.00

Flag question

Which of the following is a correct statement regarding anorexia?

Select one:

- Anorexic women are more likely to have a history of childhood sexual abuse compared to depressed women
- A significant heritability exists for bulimia but not anorexia
- The concordance rate for monozygotic twins is higher than dizygotic twins
- Persistent preoccupation with eating is a characteristic feature
- Social class distribution of prevalence of anorexia is even

Explanation:

Concordance of monozygotic:dizygotic = 65%:32%. There is no excess of childhood physical or sexual abuse in anorexia nervosa compared to psychiatric controls. Patients with anorexia nervosa have body image distortion (dread of fatness, overvalued ideas, imposed low weight threshold).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 411.

The correct answer is: The concordance rate for monozygotic twins is higher than dizygotic twins

Question **5**

Not answered

Marked out of 1.00

Flag question

Which of the following is true concerning cyclothymia?

Select one:

- It is commonly associated with endocrine disturbances
- The diagnosis can be made if there is a past history of bipolar disorder
- Mood stabilisers are of no use in management
- Hypomania is the most common reason for seeking help
- It often fails to come to medical attention

Explanation:

Because cyclothymic mood swings are relatively mild and periods of mood elevation may be enjoyable (with increased activity and productivity, self-confidence, and sociability), cyclothymia frequently fails to come to medical attention. The person may often present either because of the impact of the depressive episodes on social and work situations or because of problems related to comorbid drug or alcohol misuse. If pharmacological treatment is contemplated, this usually consists of a trial of a mood stabiliser.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 348-349.

The correct answer is: It often fails to come to medical attention

Question **6**

Not answered

Marked out of 1.00

Flag question

Which of the following delusions is least likely to be associated with a delusional disorder?

Select one:

- Erotomanic delusions
- Delusions of jealousy
- Hypochondriacal delusions
- Bizarre delusions
- Persecutory delusions

Explanation:

Delusional disorder is an uncommon condition in which patients present with circumscribed symptoms of non-bizarre delusions (DSM-5 allows ‘with bizarre content’; ICD-11 does not specify), but with absence of prominent hallucinations and no thought disorder, mood disorder, or significant flattening of affect. DSM-5 subtypes include erotomanic, grandiose, jealous, persecutory, and somatic. ICD-11 states that delusions typically persist for at least 3 months and often much longer.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 230.

The correct answer is: Bizarre delusions

Question **7**

Ganser syndrome is included in ICD-11 under which of the following chapters?

Not answered

Marked out of 1.00

Flag question

Select one:

- Disorders specifically associated with stress
- Anxiety or fear-related disorders
- Disorders of bodily distress or bodily experience
- Obsessive-compulsive or related disorders
- Other specified dissociative disorders

Explanation:

Ganser syndrome is characterised by approximate answers, disorientation, clouding of consciousness, hallucinations/pseudo-hallucinations, motor disturbance, anxiety or apathy, normal activities of daily living, and sudden resolution with amnesia for the period of illness. In ICD-11, it is included under 'other specified dissociative disorders'.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 236.

Dieguez S. Ganser syndrome. Neurologic-Psychiatric Syndromes in Focus-Part II. 2018;42:1-22.

The correct answer is: Other specified dissociative disorders

Question **8**

Not answered

Marked out of 1.00

Flag question

Which of the following is not considered a core symptom of schizophrenia in ICD-11?

Select one:

- Delusions
- Hallucinations
- Disorganised thinking
- Affective flattening
- Experiences of passivity

Explanation:

Although first-rank symptoms have been somewhat deemphasised in ICD-11, a diagnosis of schizophrenia requires the presence of at least two of seven symptom categories, including at least one of four 'core' symptoms. Core symptoms are delusions; hallucinations; experiences of influence, passivity, or control; and disorganised thinking (formal thought disorder). 'Negative symptoms' (such as affective flattening, alogia or paucity of speech, avolition, asociality, and anhedonia) represent one of the seven symptom categories, but not a core category in ICD-11.

Ref: Stein DJ et al. Mental, behavioral and neurodevelopmental disorders in the ICD-11: an international perspective on key changes and controversies. BMC medicine. 2020 Dec;18(1):1-24.

International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Affective flattening

Question **9**

Mr.Z was admitted to a psychiatric ward 7 days ago. On mental state examination, he appeared

Not answered

Marked out of 1.00

Flag question

thought disordered and also had mood-incongruent delusions and hallucinations. At times, he would laugh inappropriately. What is the most appropriate diagnosis?

Select one:

- Schizophrenia
- Secondary mania
- Bipolar disorder
- Unipolar depression
- Schizoaffective disorder

Explanation:

Mood incongruity (such as inappropriate laughter, smiling, "buffoonery", or weeping) has been considered an important sign in schizophrenia, though it is not specifically included in current diagnostic systems. It was added in DSM-III and dropped in DSM-IV. 'Mood-congruent' describes a secondary symptom which is understandable in light of an abnormal mood state (e.g a severely depressed patient developing a delusion that they are in severe debt, or a manic patient developing a delusion that they are exceptionally wealthy).

Ref: Kendler KS. Phenomenology of schizophrenia and the representativeness of modern diagnostic criteria. JAMA psychiatry. 2016 Oct 1;73(10):1082-92.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 114.

The correct answer is: Schizophrenia

Question **10**

Not answered

Marked out of 1.00

Flag question

A 53-year-old lawyer is concerned about a recent change in the nature of his heart rhythm. His GP has referred him to various cardiologists who were all unable to find any abnormalities in ECG, stress test, perfusion scan, echocardiogram and a 24 hour Holter monitoring. But the lawyer feels that something sinister has been missed and continues spending many hours consulting various books, journals and internet resources to understand his problems. He has stopped working as a result. The most likely diagnosis is

Select one:

- Conversion disorder
- PTSD
- Hypochondriasis
- Munchausen disorder
- Somatisation disorder

Explanation:

Hypochondriasis is the preoccupation with the fear of having a serious disease, which persists despite negative medical investigations and appropriate reassurance, with subsequent distress and impaired function.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 870.

The correct answer is: Hypochondriasis

Question **11**

Not answered

Marked out of 1.00

Flag question

A 42-year-old man believes that he has contracted a serious infectious condition when he travelled to Burma, where his wife met with an accident. Since then he has seen multiple physicians and had numerous tests including the most sophisticated microbiological investigations but he is not convinced of being healthy. Even at times when he is not preoccupied with healthcare services, he continues to maintain that he carries a serious infection. He also continues to feel low as a result for the last 1 year. What is the most appropriate diagnosis?

Select one:

- Body Dysmorphic Disorder
- Somatoform disorder
- Hypochondriasis with fair insight
- Factitious disorder
- Hypochondriasis with poor insight

Explanation:

Hypochondriasis is an anxiety state. A patient with hypochondriasis may hold a belief as an overvalued idea bordering on delusional intensity. The patient in question here seems to hold the belief with absolute conviction; ICD-11 allows a specifier "with poor insight" for an individual who, either most or all of the time, is "convinced that the disorder-specific beliefs are true and they cannot accept an alternative explanation for their experience. The lack of insight exhibited by the individual does not vary markedly as a function of anxiety level".

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 870.

The correct answer is: Hypochondriasis with poor insight

Question **12**

Not answered

Marked out of 1.00

Flag question

Which of the following is a characteristic feature of narcolepsy?

Select one:

- Catalepsy
- Sleep walking
- Sleep paralysis
- Apnea
- Nocturnal compulsions

Explanation:

Classic symptoms of narcolepsy include: excessive daytime sleepiness; cataplexy (sudden loss of muscle tone during wakefulness that is evoked by strong, mainly positive, emotions); sleep paralysis (sometimes up to 10 minutes long); and hypnagogic hallucinations (vivid hallucinations on falling asleep) or hypnopompic hallucinations (hallucinations when the individual is waking up).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 450.

Kornum BR et al. Narcolepsy. Nature reviews Disease primers. 2017 Feb 9;3(1):1-9.

The correct answer is: Sleep paralysis

Question **13**

Not answered

Marked out of 1.00

Flag question

Which of the following is not a common clinical feature of mania?

Select one:

- Reduced need for sleep
- Irritability
- Memory impairment
- Physical overactivity
- Elevated mood out of keeping with the circumstances

Explanation:

Clinical features of mania:

- Elevated mood (out of keeping with circumstances)
- Increased energy which may manifest as overactivity and/or reduced need for sleep
- Formal thought disorder which may manifest as pressured speech, flight of ideas, and/or racing thoughts
- Increased self-esteem, evident as over-optimistic ideation, grandiosity, reduced social inhibitions, and/or over-familiarity
- Reduced attention and increased distractibility
- Tendency to engage in behaviour that may have serious consequences (e.g. spending recklessly, inappropriate sexual encounters)
- Other behavioural manifestations, including excitement, irritability, aggressiveness, and suspiciousness
- Marked disruption of work, usual social activities, and family life

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 320-321.

The correct answer is: Memory impairment

Question **14**

Not answered

Marked out of 1.00

Flag question

The term schizoaffective psychosis was coined by

Select one:

- Kretschmer
- Griesinger
- Kraepelin
- Kasanin
- Kanner

Explanation:

The term schizoaffective psychosis was introduced by the Russian-born American psychiatrist Jacob Kasanin in 1933. The earlier concepts of schizoaffective disorder regarded this diagnosis as a good prognosis schizophrenia, with psychoanalytic explanations.

Ref: Kasanin JA. The acute schizoaffective psychoses. American Journal of Psychiatry. 1933 Jul;90(1):97-126.

The correct answer is: Kasanin

Question **15**

Not answered

Marked out of 1.00

Flag question

Which of the following is true about patients with anorexia nervosa?

Select one:

- The illness has less mortality rate than schizophrenia
- The most common cause for presentation is comorbid personality disorder
- The incidence is 35% in the monozygotic twins of affected patients
- The incidence is 15% in the dizygotic twins of affected patients
- There is no association between social class and development of anorexia

Explanation:

Anorexia nervosa has the highest rate of mortality among all mental disorders. Though anorexia nervosa is found globally, it is considered more prevalent in some Western societies where cultural influences promote thinness as the ideal body shape. However, there is no evidence that differences such as ethnicity or socioeconomic status make it more likely that that a member of any one group will have an eating disorder. The genetic concordance of anorexia nervosa is MZ:DZ = 65%:32%.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 410-411.

National Institute for Health and Clinical Excellence. NICE guideline Eating Disorders: recognition and treatment 2017.

The correct answer is: There is no association between social class and development of anorexia

Question **16**

Not answered

Marked out of 1.00

Flag question

Which of the following is not indicative of excessive preoccupation with body weight or shape in anorexia nervosa?

Select one:

- Repeatedly checking one’s reflection in mirrors
- Refusal to have mirrors at home
- Avoiding tight-fitting clothes
- Refusing to know one’s weight
- Avoiding knowing the calorie content of food

Explanation:

Anorexia nervosa is characterised by significantly low body weight for the individual’s height, age, and developmental stage that is not due to another health condition or to the unavailability of food.

Low body weight or shape is central to the person's self-evaluation or is inaccurately perceived to be normal or even excessive. Preoccupation with weight or shape, when not explicitly reported, may be manifested by behaviours such as repeatedly checking body weight using scales; repeatedly checking one's body shape using tape measures or reflection in mirrors; constantly monitoring the calorie content of food or searching for information on how to lose weight; or by extreme avoidant behaviours, such as refusal to have mirrors at home, avoidance of tight-fitting clothes, or refusal to know one's weight or purchase clothing with specified sizing.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Avoiding knowing the calorie content of food

Question **17**

Not answered

Marked out of 1.00

Flag question

The progression of post-traumatic stress disorder (PTSD)

Select one:

- Consistently follows a linear course
- Shows initial low symptom levels with an increase in symptoms over time
- Evolves into chronic distress with high symptom levels for most people
- Fluctuates over time and is not static
- Is concretely determined in the acute phase following trauma

Explanation:

The understanding that PTSD follows a linear course has been challenged in recent years by evidence that the severity of the disorder fluctuates over time, that it can worsen or remit, and that this pattern can keep recurring, with the result that one's PTSD status is not static. Studies have demonstrated a resilient class which consistently shows few PTSD symptoms, a recovery class with initial distress followed by gradual remission, a delayed reaction class with initial low symptom levels but increased symptoms over time, and a chronic distress class with consistently high PTSD levels. There is also evidence that the PTSD syndrome develops over time. In the acute phase after trauma, PTSD symptoms appear more loosely interconnected, while they become more closely related with the known factors (e.g., re-experiencing, active avoidance) as time progresses.

Ref: Bryant RA. Post-traumatic stress disorder: a state-of-the-art review of evidence and challenges. World Psychiatry. 2019 Oct;18(3):259-69.

The correct answer is: Fluctuates over time and is not static

Question **18**

Not answered

Marked out of 1.00

Flag question

A 31-year-old woman 6 days postpartum has not been sleeping well. She has been wandering in the middle of the night. She is ignoring her baby, and reports that "My child is a sign of a terrible disaster about to happen to the world". Which of the following is the most likely ICD-11 diagnosis?

Select one:

- Post traumatic stress disorder
- Mental disorder associated with the puerperium, with psychotic symptoms
- Major depressive disorder with psychotic features
- Delirium
- Postpartum blues

Explanation:

This scenario depicts postpartum psychosis. Postpartum psychosis has three common clinical presentations: prominent affective symptoms (mania or depression with psychotic symptoms), schizophreniform disorder, or acute organic psychosis. ICD-11 recommends using this term for significant mental disorders that are commencing within about 6 weeks after delivery that include delusions, hallucinations, or other psychotic symptoms. This designation should not be used for the so-called postpartum blues.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 494-495.

The correct answer is: Mental disorder associated with the puerperium, with psychotic symptoms

Question **19**

Not answered

Marked out of 1.00

Flag question

Most children with selective mutism also meet diagnostic criteria for which of the following disorders?

Select one:

- Depression
- Asperger syndrome
- Generalised anxiety disorder
- Autism
- Social anxiety disorder

Explanation:

Selective mutism is characterised by the persistent failure to speak in specific settings (e.g. school) where there is an expectation to speak. It begins after normal speech has been acquired, and there is full use of language at home/with family. Patients may remain wholly silent or near-silent, in some cases only whispering in a school setting. In rare cases, a child is mute at home but not in school. Social anxiety is almost always present.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 687.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 201.

The correct answer is: Social anxiety disorder

Question **20**

Not answered

Marked out of 1.00

Flag question

The term schizophrenia was coined by

Select one:

- Kahlbaum
- Freud
- Kraepelin
- Jung
- Bleuler

Explanation:

Eugene Bleuler coined the term *schizophrenia* (1911), which replaced *dementia praecox* in the literature. Deriving it from the ancient Greek words 'schizo' ('I split') and 'phren' ('mind'), he chose the term to express the presence of schisms among thought, emotions, and behaviour in patients with the disorder.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1099.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 182.

The correct answer is: Bleuler

Question **21**

Not answered

Marked out of 1.00

Flag question

Which of the following is true regarding schizophrenia subtypes in ICD-11?

Select one:

- Catatonia is considered to be a subtype of schizophrenia
- Subtypes of schizophrenia include paranoid and hebephrenic
- Schizophrenia subtypes have shown predictive validity
- All subtypes of schizophrenia have been eliminated
- None of the listed options

Explanation:

A significant change in ICD-11 is the elimination of all subtypes of schizophrenia (e.g., paranoid, hebephrenic, catatonic), due to their lack of predictive validity or utility in treatment selection. In lieu of the subtypes, a set of dimensional descriptors has been introduced. In ICD-10, catatonia was included as one of the subtypes of schizophrenia and as one of the organic disorders (i.e., organic catatonic disorder). In recognition of the fact that the syndrome of catatonia can occur in association with a variety of mental disorders, a new diagnostic grouping for catatonia (at the same hierarchical level as mood disorders, anxiety and fear-related disorders, etc.) has been added in ICD-11.

Ref: Reed GM et al. Innovations and changes in the ICD-11 classification of mental, behavioural and neurodevelopmental disorders. World Psychiatry. 2019 Feb;18(1):3-19.

The correct answer is: All subtypes of schizophrenia have been eliminated

Question **22**

Not answered

Marked out of 1.00

Flag question

A 40 year old lady had an episode of depression 4 years ago. For last 4 weeks she is low in mood, socially isolating herself, and fears going out of her home. She fears that she will get a panic attack if she goes out. She is quite tearful and has lost much sleep. The most appropriate working diagnosis is thought to be depressive relapse, and other differential diagnoses are excluded in the above scenario before arriving at the diagnosis. This is guided by which of the following principles of classification?

Select one:

- Atheoretical principle
- Hierarchical principle
- Descriptive principle
- Multiaxial principle

- Operationalised principle

Explanation:

The ideal diagnostic system labels diseases according to aetiology. The aetiology of most mental disorders in unknown, so we tend towards a diagnostic system based on common clinical features, shared natural history, common treatment response, or a combination of all three. Diagnosis leads to the consideration of individual diseases as members of groups contained within a hierarchy – a form of classification system.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 10.

The correct answer is: Hierarchical principle

Question **23**

Not answered

Marked out of 1.00

Flag question

A mother wants to know the likelihood that her teenage son who has been smoking cannabis for 6 months will develop schizophrenia. The correct answer is

Select one:

- Nil increase in risk
- 9 - 10 times
- 7 - 8 times
- 5 - 6 times
- 2 - 4 times

Explanation:

Cannabis use, especially in adolescence, confers a twofold increased risk of later schizophrenia. The highest risk is for younger adolescents (fourfold increased risk for those using cannabis by age 15).

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

Hollis C. Schizophrenia in children and adolescents. BJPsych Advances. 2015 Sep;21(5):333-41.

The correct answer is: 2 - 4 times

Question **24**

Not answered

Marked out of 1.00

Flag question

You are seeing an elderly man with a history of low mood. He says he was feeling well until 18 months ago but progressively became low in his mood since then. He also complains of becoming more forgetful over the last year. However, he has no problem sleeping and appetite remains reasonably stable. Few months ago, he woke up suddenly with a limp and has been suffering from it since then. What is the most likely diagnosis?

Select one:

- Alzheimer disease
- Multiple sclerosis
- Stroke
- Lewy body dementia
- Depression

Explanation:

Psychiatric sequelae of stroke include cognitive disorders and post-stroke depression.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 176.

The correct answer is: Stroke

Question **25**

Not answered

Marked out of 1.00

Flag question

Which of the following personality types is classified as a Cluster A personality disorder?

Select one:

- Obsessive-compulsive personality disorder
- Histrionic personality disorder
- Avoidant personality disorder
- Schizotypal personality disorder
- Antisocial personality disorder

Explanation:

DSM-5 uses three broad clusters to organise the categories of personality disorders:

- Cluster A (odd/eccentric): paranoid, schizoid, schizotypal
- Cluster B (emotional/dramatic): antisocial, histrionic, narcissistic, borderline
- Cluster C (fearful/anxious): avoidant, dependent, obsessive-compulsive

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 561.

The correct answer is: Schizotypal personality disorder

Question **26**

Not answered

Marked out of 1.00

Flag question

A 12 yr old boy is found to be skipping school repeatedly and using drugs with his friends. He is most likely to have which of the following diagnosable psychiatric conditions?

Select one:

- Asperger syndrome
- Oppositional defiant disorder
- Autism
- ADHD
- Conduct disorder

Explanation:

Conduct disorder is characterised by a repetitive and persistent pattern of **antisocial, aggressive or defiant behaviours that defy age-appropriate societal norms or rules.** Depending on the age/stage of the child, this may include: aggression/cruelty to people and/or animals, destruction of property, bullying, deceitfulness, lying/blaming others, theft, fire setting, truancy/running away from

home, severe provocative or disobedient/defiant behaviour, forced sexual activity, use of a weapon.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 664.

The correct answer is: Conduct disorder

Question **27**

Not answered

Marked out of 1.00

Flag question

Mr X is a 31-year-old gentleman who has suffered three manic episodes and 1 episode of depression in the past. He is already on antipsychotics and antidepressants. When reviewed in the out-patient clinic, he complains of low mood, frequent crying spells, poor concentration and lack of enjoyment in things he used to do. Which one of the following is the most appropriate medication for his condition?

Select one:

- Amitriptyline
- Lamotrigine
- Venlafaxine
- Sertraline
- Mirtazapine

Explanation:

NICE guidelines recommend the following for bipolar depression:

If depression is **moderate-severe** and patient is **not taking medication to treat bipolar**, offer:

1. **Fluoxetine combined with olanzapine (OFC)**
2. **Quetiapine** on its own
3. If patient prefers, **either olanzapine or lamotrigine** on its own
4. If **no response to either of first two options**, consider **lamotrigine** on its own

If depression is **moderate-severe** and patient is **already taking lithium**, **check lithium level and optimize; if lithium is maximized, add olanzapine+fluoxetine, add quetiapine, add olanzapine, or add lamotrigine**

If depression is **moderate-severe** and patient is **already taking valproate**, considering inc to **max dose within the therapeutic range**; if already done, **add olanzapine+fluoxetine, add quetiapine, add olanzapine, or add lamotrigine**

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Lamotrigine

Question **28**

Not answered

Marked out of 1.00

Flag question

According to ICD-11, acute and transient psychotic disorder is characterised by an acute onset of a clear psychotic state from a non-psychotic state within a period of:

Select one:

- 2 days
-

1 week

- 4 weeks
- 2 weeks
- 1 day

Explanation:

Acute and transient psychotic disorder is characterised by acute onset of psychotic symptoms that emerge without a prodrome and reach their maximal severity within two weeks. Symptoms may include delusions, hallucinations, disorganisation of thought processes, perplexity or confusion, and disturbances of affect or mood. Catatonia-like motor disturbances may be present. Symptoms typically change rapidly, both in nature and intensity, from day to day, or even within a single day. The duration of the episode does not exceed 3 months, and most commonly lasts from a few days to 1 month.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: 2 weeks

Question **29**

Not answered

Marked out of 1.00

Flag question

A mother complains that her 20-year-old son is upsetting his employers by being very rigid in his duties and not able to delegate tasks. He also has difficulties in discarding his materials and hoards items including old newspapers in his office. What is the most likely diagnosis?

Select one:

- Schizoid personality disorder
- Obsessive-compulsive disorder
- Normal presentation
- Obsessive-compulsive personality disorder
- Schizophrenia

Explanation:

Clinical features of patients with obsessive compulsive personality disorder include reluctance to delegate; perfectionism interfering with completion of tasks; miserly spending and hoarding of money; inability to discard worthless objects; excessive devotion to work and productivity; preoccupation with details, rules, lists, order, schedules; inflexibility about morality and ethics; rigidity and stubbornness.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 566.

The correct answer is: Obsessive-compulsive personality disorder

Question **30**

Not answered

Marked out of 1.00

Flag question

Which one of the following is not categorised as an autism spectrum disorder in DSM-5?

Select one:

- ADHD
- Asperger syndrome
- Autism

- Childhood disintegrative disorder
- Pervasive Developmental Disorder-NOS

In DSM-5 the previous categories of Autistic Disorder, Asperger's Disorder and Pervasive Developmental Disorder-NOS and Childhood Disintegrative Disorder (Hellers syndrome) were replaced by the diagnosis of Autism Spectrum Disorder. Additionally, the DSM-5 also reduced social-related elements of autism into social communication impairment and repetitive/restricted behaviours.

The correct answer is: ADHD

Question **31**

Not answered

Marked out of 1.00

Flag question

Which of the following is a poor prognostic factor for schizophrenia in terms of relapse/recovery from the illness?

Select one:

- Late onset
- Higher premorbid functioning
- Presence of affective symptoms
- Male sex
- Acute onset

Explanation:

Poor prognostic factors in schizophrenia include: poor premorbid adjustment/functioning; insidious onset; young onset (childhood or adolescence); cognitive impairment; enlarged ventricles; male sex; negative symptoms (not mood symptoms, which are a positive prognostic factor).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 207.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 350.

The correct answer is: Male sex

Question **32**

Not answered

Marked out of 1.00

Flag question

Which one of the following can be a discriminating symptom while making a clinical diagnosis?

Select one:

- Memory problems
- Poor concentration
- Thought withdrawal
- Panic attacks
- Anhedonia

Explanation:

Thought withdrawal is a first rank symptom for a diagnosis of schizophrenia. This is considered to be a

discriminating symptom while making a clinical diagnosis. First rank symptoms of schizophrenia are useful because they occur reasonably often in schizophrenia and more rarely in other disorders, and it is not too difficult to tell whether they are present or not.

First rank symptoms can be categorised as follows:

- Auditory hallucinations (voices heard arguing, thought echo, running commentary)
- Delusions of thought interference (thought insertion, thought withdrawal, thought broadcasting)
- Delusions of control (passivity of affect, passivity of impulse, passivity of volitions, somatic passivity)
- Delusional perception (a primary delusion of any content that is reported by the patient as having arisen, following the experience of a normal perception)

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 78.

The correct answer is: Thought withdrawal

Question **33**

Not answered

Marked out of 1.00

Flag question

What percentage of non-schizophrenic patients with psychosis exhibit first rank symptoms of schizophrenia?

Select one:

- 40-45%
- 25-35%
- 5-7%
- 10-20%
- 50-60%

Explanation:

Kurt Schneider enumerated a number of symptoms which he believed were of first-rank importance in differentiating schizophrenia from related illnesses. According to the International Pilot Study of Schizophrenia, 58% of patients with acute schizophrenia exhibited at least one first rank symptom. Generally, the prevalence of first-rank symptoms in schizophrenia is reported to range between 25 and 88%, and in a Cochrane diagnostic test review it was 57%. However, at least 20% of people with schizophrenia never exhibit a first rank symptom while almost 10% of non-schizophrenic patients exhibit them. This value can be higher (19%, per Soares-Weiser et al., 2015) in studies that specifically examine FRS.

Ref: Mitchell AJ. First-rank symptoms: a first-rank diagnostic test?. BJPsych Advances. 2015 May;21(3):147-9.

Soares-Weiser , K, Maayan , N, Bergman , H et al (2015) First rank symptoms for schizophrenia (Review). Cochrane Library , issue 1 (http://onlinelibrary.wiley.com/doi/10.1002/14651858.CD010653.pub2/pdf).

The correct answer is: 10-20%

Question **34**

Not answered

Marked out of 1.00

A 25-year-old man has experienced an irrational fear of darkness since childhood. He is not distressed by this currently and does not take special measures to avoid dark spaces. Which of the following is true?

Flag question

Select one:

- He has a specific phobia as he has an irrational fear
- He has no specific phobia as fear of darkness is common
- He has a specific phobia with loss of insight
- He has no specific phobia as he does not have an avoidance behaviour
- He has a specific phobia as he has it since childhood

Explanation:

In phobias of all kinds, there is an avoidance of the feared situation. Specific phobias are restricted to specific situations such as proximity to certain animals, natural phenomenon such as heights, thunder, darkness, flying, being trapped in closed spaces, urinating or defecating in public toilets, eating certain foods, dentistry, or the sight of blood or injury.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 69.

The correct answer is: He has no specific phobia as he does not have an avoidance behaviour

Question **35**

Not answered

Marked out of 1.00

Flag question

A 26-year-old woman has fears of being judged negatively by other people. This is suggestive of

Select one:

- Borderline personality disorder
- Avoidant personality disorder
- Obsessive-compulsive personality disorder
- Dependent personality disorder
- Histrionic personality disorder

Explanation:

Clinical features of patients with avoidant personality disorder include avoiding jobs with significant interpersonal contact for fear of criticism or rejection; unwillingness to get involved unless certain of being liked; restraint in relationships because of fear of being ridiculed; preoccupation with being criticized or rejected; social inhibition because of feelings of inadequacy; viewing self as inferior; reluctance to take risks because of possible embarrassment.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 565.

The correct answer is: Avoidant personality disorder

Question **36**

Not answered

Marked out of 1.00

Flag question

Which of the following culture-bound syndromes refer to fear of turning into a cannibal?

Select one:

- Windigo
- Koro
- Susto
- Piblokto

- Latah

Explanation:

Witiko psychosis (or Windigo) refers to a person developing the delusion of transforming into a cannibalistic monster and craving human flesh under the threat of starvation in severely cold weather. (Note: The term 'witiko' is more appropriate to capture this condition, and the Western interpretation of what is described as Windigo being attributed to the ethnic groups of Cree and Metis people has been disputed.)

Ref: Tseng WS. From peculiar psychiatric disorders through culture-bound syndromes to culture-related specific syndromes. Transcultural psychiatry. 2006 Dec;43(4):554-76.

Carlson ND. Reviving Witiko (Windigo): An ethnohistory of "cannibal monsters" in the Athabasca district of northern Alberta, 1878–1910. Ethnohistory. 2009;56(3):355-94.

The correct answer is: Windigo

Question **37**

Not answered

Marked out of 1.00

Flag question

A patient with long-standing anxiety being managed by her GP was recently seen by a psychiatrist who made changes in her medications. She presents to the Accident and Emergency Unit with hallucinations of her joints being moved, as well as nausea, agitation, insomnia, and anxiety. What is the diagnosis?

Select one:

- Antidepressant withdrawal
- Benzodiazepine intoxication
- Benzodiazepine withdrawal
- Alcohol intoxication
- Acute transient psychosis

Explanation:

Symptoms of benzodiazepine withdrawal include anxiety, insomnia, tremor, agitation, headache, nausea, sweating, depersonalisation, seizures, and delirium. Sometimes grand mal seizures can occur. Kinaesthetic hallucinations can occur in schizophrenia or in organic states such as alcohol intoxication and during benzodiazepine withdrawal. Kinaesthetic hallucinations affect the muscles and joints, and the patient feels that their limbs are being twisted, pulled, or moved.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 638.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 34.

The correct answer is: Benzodiazepine withdrawal

Question **38**

Not answered

Marked out of 1.00

A sportsperson is found to use excessive opioid analgesics to help symptoms of pain following intense training. He abuses opioids in doses that are much higher than the prescribed amount and refuses to stop them despite medical advice. In ICD-11, this condition is described in which of the following chapters?

Flag question

Select one:

- Impulse control disorders
- Disruptive behaviour or dissocial disorders
- Disorders specifically associated with stress
- Personality disorders and related traits
- Disorders due to substance use or addictive behaviours

Explanation:

In ICD-11, 'Disorders due to substance use or addictive behaviours' are mental and behavioural disorders that develop as a result of the use of predominantly psychoactive substances, including medications, or specific repetitive rewarding and reinforcing behaviours. For disorders due to use of opioids, this includes harmful pattern of use of opioids and opioid dependence.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Disorders due to substance use or addictive behaviours

Question **39**

Not answered

Marked out of 1.00

Flag question

Which term refers to 'discrete episodes of anterograde amnesia that occur in association with alcohol intoxication'?

Select one:

- Pathological intoxication
- Wernicke encephalopathy
- Korsakoff syndrome
- Alcoholic dementia
- Alcoholic blackouts

Explanation:

Alcohol-induced amnesia or blackouts or 'palimpsest' are not included in DSM-5/ICD-11. This is characterised by discrete episodes of anterograde amnesia that occur in association with alcohol intoxication. Patients will report a gap in their memory lasting for several hours, with global or partial amnesia for their actions during that time. The patient's behaviour is usually characteristic of their normal behaviour when intoxicated. There is a breakdown in transfer of information from short-term to long-term storage which occurs while immediate (very brief short-term) and remote (long-term; formed prior to intoxication) memory abilities remain intact. Alcohol blocks the consolidation of new memories into old memories, a process that is thought to involve the hippocampus and related temporal lobe structures. Alcohol facilitates faster forgetting. Rapid forgetting is a hallmark of hippocampal dysfunction.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 600.

Hermens DF, Lagopoulos J. Binge drinking and the young brain: A mini review of the neurobiological underpinnings of alcohol-induced blackout. Frontiers in psychology. 2018 Jan 19;9:12.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 277-278.

The correct answer is: Alcoholic blackouts

Question **40**

Not answered

Marked out of 1.00

Flag question

A patient suffers from florid racing of thoughts, decreased sleep, high energy levels and believes aliens are putting thoughts into his brain. He also believes his nail bed is made soft by 'Bull-bull bird'. His history suggests that he always had affective and psychotic disturbances occurring simultaneously, and that both have been equally prominent. Which of the following chapters will his diagnosis fall under in ICD-11?

Select one:

- Neurocognitive disorders
- Schizophrenia or other primary psychotic disorders
- Mood disorders
- Disorders specifically associated with stress
- Anxiety or fear-related disorders

Explanation:

In ICD-11, schizoaffective disorder is characterised by schizophrenic and affective symptoms that are simultaneously present for at least 1 month, and both are equally prominent. Schizoaffective disorder is in the 'schizophrenia or other primary psychotic disorders' grouping.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 228.

International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Schizophrenia or other primary psychotic disorders

Question **41**

Not answered

Marked out of 1.00

Flag question

Which of the following is true concerning brain injury and schizophrenia?

Select one:

- Genetic predisposition to schizophrenia has no influence on vulnerability for psychosis after brain injury
- Brain injury increases risk of non-affective psychosis more than affective psychosis
- Psychosis related to brain injury is usually acute in onset
- Brain injury markedly increases likelihood of developing schizophrenia
- Children are more susceptible to schizophrenia following brain injury than adults

Explanation:

A schizophrenia-like psychotic disorder after brain injury occurs relatively rarely 1-5 years after injury and is associated with frontal and temporal damage. Persecutory delusions and auditory hallucinations are the more frequent postinjury psychotic sequelae, with low rates of negative symptoms (e.g., anhedonia), diverging from the symptom profile common to schizophrenia. Children have less psychopathology after brain injury than adults due to increased brain plasticity.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 173.

Ponsford J et al. Epidemiology and natural history of psychiatric disorders after TBI. The Journal of neuropsychiatry and clinical neurosciences. 2018 Oct;30(4):262-70.

The correct answer is: Brain injury increases risk of non-affective psychosis more than affective psychosis

Question **42**

Not answered

Marked out of 1.00

Flag question

A 35-year-old man with intellectual disability works part-time doing cleaning tasks at a local shopping complex. He lives on his own but receives regular visits from his family who often bring baked-goods and other small gifts. His language is basic but comprehensible. What is his level of intellectual disability?

Select one:

- Borderline
- Mild
- Moderate
- Severe
- Profound

Explanation:

Level of intellectual disability	Level of functioning
Mild	Often struggle with academic skills and complex language comprehension Most are capable of basic self-care and domestic and practical activities Most are able to live relatively independently Capable of finding employment as adults, but may need appropriate support
Moderate	Capacity for academic skills and language comprehension varies, usually limited to basic skills Some can manage basic self-care and domestic and practical activities Most need significant ongoing support to live independently and maintain employment as adults
Severe	Very limited capacity for language and ability to gain academic skills Often accompanied by motor impairments Daily support in a supervised environment is usually required Some may develop basic self-care skills with intensive support and training
Profound	Very limited communication ability Potential to gain academic skills is limited to basic concrete skills Motor and sensory impairments usually present Daily support in a supervised care environment usually required

Adapted from Williams et al. 2018



Ref: Williams F et al. Alcohol use disorders in people with intellectual disability. BJPsych Advances. 2018 Jul;24(4):264-72.

The correct answer is: Mild

Question **43**

Not answered

Marked out of 1.00

According to DSM-5 criteria, the core symptoms of ADHD must have their onset

Select one:

- Before 12 years of age
-

Flag question

Before 7 years of age

Before 9 years of age

Before 3 years of age

Before 18 years of age

Explanation:

According to the older DSM-IV criteria, to meet the diagnosis of ADHD, some symptoms must be present before the age of 7 years although ADHD is not diagnosed in many children until they are older than 7 years when their behaviours cause problems in school and other places.

DSM-5 has introduced changes in the diagnosis of ADHD: symptoms can now occur before age 12 rather than before age 7; several symptoms now need to be present in more than one setting rather than just the impairment criteria being satisfied in more than one setting; new descriptions are now added to capture ADHD features in later ages. For individuals aged 17 or older, only 5 symptoms are required for a diagnosis instead of the usual 6 needed in children.

Ref: http://www.cdc.gov/ncbddd/adhd/diagnosis.html

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

The correct answer is: Before 12 years of age

Question **44**

Not answered

Marked out of 1.00

Flag question

A 28-year-old man presents for the seventh time to his GP with abdominal pain. When confronted, he admits to faking symptoms. He says "I know this is wrong, but I don't know why I do this". Which of the following is the most likely diagnosis for this patient?

Select one:

conversion disorder

somatization disorder

factitious disorder

hypochondriasis

malingering

Explanation:

In factitious disorder, patients intentionally falsify their symptoms and past history and fabricate signs of physical or mental disorder, with the primary aim of obtaining medical attention and treatment.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 876.

The correct answer is: factitious disorder

Question **45**

Not answered

Marked out of 1.00

Flag

Diagnostic features of myalgic encephalomyelitis include all of the following EXCEPT

Select one:

Duration >6 months

Easy fatiguability after minimal effort

question

- Fatigue alleviated by prolonged rest
- Muscular aches and pains
- Poor (unrefreshing) sleep

Explanation:

Chronic fatigue syndrome (CFS) criteria US-CDCP (1994) suggest that to diagnose CFS (or myalgic encephalomyelitis), patients must have persistent or relapsing unexplained chronic fatigue. The fatigue must last for at least 6 months and should not be alleviated by rest.

Criteria	Definition of Chronic Fatigue Syndrome (US Center for Disease Control and Prevention, 1994)
Main criteria	Persistent or relapsing fatigue of 6 months duration or more Fatigue being not explained by any concurrent somatic or psychiatric condition Fatigue is new or has a defined onset Fatigue is not due to ongoing exertion Fatigue is not relieved by rest
Additional criteria (at least 4)	Impaired memory and/or concentration that causes a reduction in previous level of activity Sore throat Tender cervical and/or axillary lymph nodes Muscle pain Multi-joint pain New headaches Unrefreshing sleep Post-exertional malaise
Exclusion criteria	Medical condition explaining fatigue (e.g. untreated hypothyroidism, sleep apnoea, narcolepsy) Previously diagnosed medical condition whose resolution has not been documented Past or current diagnosis of MDD or bipolar disorder, schizophrenia, dementia, delusional disorder, anorexia nervosa, bulimia nervosa, substance use disorder within 2 years before onset Severe obesity (BMI > 45) Unexplained physical exam/lab/imaging abnormality that strongly suggests exclusionary condition

SPMM COURSE

Ref: Prochalska C et al. Chronic fatigue syndrome: An update for psychiatrists. Open Journal of Psychiatry 2012 2:40-8.

The correct answer is: Fatigue alleviated by prolonged rest

Question **46**

Not answered

Marked out of 1.00

Flag question

Fugue state has been recognised as a type of

Select one:

- Situation specific psychogenic amnesia
- False memory syndrome
- Dissociative amnesia
- Post traumatic amnesia
- Transient global amnesia

Explanation:

In dissociative amnesia with dissociative fugue, there is loss of personal identity and sudden travel away from home, work, or significant others for an extended period of time (days or weeks). A new identity may be assumed.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11;

World Health Organization, 2020)

The correct answer is: Dissociative amnesia

Question **47**

Not answered

Marked out of 1.00

Flag question

Dream-like (oneiroid) states with visual hallucinations are associated with which of the following features?

Select one:

- Negative symptoms
- Paranoid ideations
- Catatonic features
- Features of formal thought disorder
- Hypomanic features

Explanation:

Oneiroid syndrome (OS), or dream-like fantastic delusional derangement of consciousness, is characterized by a kaleidoscopic quality of psychopathological experiences, wherein reality, illusions and hallucinations are merged into one. It is typically accompanied by motor and, in particular, catatonic disturbances.

Excerpt from Kapstan A, Miodownick C, Lerner V. Oneiroid syndrome: a concept of use for western psychiatry. Israel Journal of Psychiatry. 2000;37(4):278.

Ref: Alekseeva AG, Tiganov AS. The typology of oneiroid-catatonic states in schizophrenia. Zhurnal Nevrologii i Psikhiatrii Imeni SS Korsakova. 2014 Jan 1;114(12):36-41.

The correct answer is: Catatonic features

Question **48**

Not answered

Marked out of 1.00

Flag question

Global assessment of functioning forms which of the following axes in DSM-IV?

Select one:

- One
- Two
- Three
- Five
- Four

The multi-axial System of DSM-IV used 5 axes. Axis I - Clinical Disorders. Axis II - Personality Disorders/ Mental Retardation. Axis III - General Medical Conditions. Axis IV - Psychosocial and Environmental Problems. Axis V - Global Assessment of Functioning. Note that DSM-5 has moved to a nonaxial system for diagnosis, with separate notations for crucial psychosocial factors (formerly Axis IV) and disability or impairment (formerly Axis V).

The correct answer is: Five

Question **49**

Not answered

Marked out of 1.00

Flag question

A 5-year boy presents to your clinic with his parents. He has difficulty relating to others in a social setting and has poor non-verbal social skills. However, he has a normal development of speech and language. What is his most likely diagnosis?

Select one:

- Rett syndrome
- Kanner syndrome
- Autism spectrum disorder
- Heller syndrome
- DiGeorges syndrome

Explanation:

Autism spectrum disorder is characterised by persistent deficits in the ability to initiate and to sustain reciprocal social interaction and social communication, and by a range of restricted, repetitive, and inflexible patterns of behaviour, interests, or activities that are clearly atypical or excessive for the individual's age and sociocultural context. Individuals along the spectrum exhibit a full range of intellectual functioning and language abilities.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Autism spectrum disorder

Question **50**

Not answered

Marked out of 1.00

Flag question

Asperger's syndrome is characterised by all of the following except

Select one:

- Lack of friendships
- Restrictive, repetitive or stereotyped patterns of behaviour
- Impaired reciprocal social interaction
- Impaired intellectual ability and syntactical speech
- Lack of awareness of the feeling of others

Explanation:

Asperger syndrome has been described as 'mind blindness'. It is characterised by severe persistent impairment in reciprocal social interactions, repetitive behaviour patterns, and restricted interests. IQ and language are normal or, in some cases, superior. Social deficits commonly manifest in adolescence or early adulthood when the individual experiences difficulty with intimate relationships. Mild motor clumsiness and a family history of autism may be present.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 820.

The correct answer is: Impaired intellectual ability and syntactical speech

Question **51**

The concept that overwhelming stress could produce mental illness led to the introduction of which of

Not answered

Marked out of 1.00

Flag question

the following terms in the early 20th century?

Select one:

- Somatisation
- Alexithymia
- Moral insanity
- War neurosis
- Precocious degeneration

Explanation:

Trauma-related symptoms among veterans of military engagement have been documented at least since the time of the ancient Greeks. Since the third edition of the DSM in 1980, this condition has been known as PTSD, but the name has changed repeatedly over the past century, including shell shock, war neurosis, and soldier's heart.

Ref: Chekroud AM et al. PTSD and the War of Words. Chronic Stress. 2018 Apr;2:1-5.

The correct answer is: War neurosis

Question **52**

Not answered

Marked out of 1.00

Flag question

A woman with borderline personality disorder is keen to know which of her symptoms will get better in the natural course of her life even if she does not take treatment. The correct answer will be

Select one:

- Identity disturbance
- Feelings of abandonment
- Impulsivity
- Intensity of interpersonal relationships
- Affective instability

Explanation:

The course of core features of borderline personality disorder persists throughout adulthood, such as affective symptoms (chronic dysphoria, anger, and feelings of emptiness) and interpersonal symptoms related to fears of abandonment. Whereas impulsivity, self-injurious, and suicidal behaviour decrease during adulthood.

Ref: Videler AC et al. A life span perspective on borderline personality disorder. Current psychiatry reports. 2019 Jul;21(7):1-8.

The correct answer is: Impulsivity

Question **53**

Not answered

Marked out of 1.00

Flag

Which of the following classificatory systems require 6 months duration with at least 2 out of 5 described symptoms to be present for a diagnosis of schizophrenia?

Select one:

- DSM-5

question

- Research Diagnostic Criteria ICD10
- ICD-11
- NIMH Research Domain Criteria
- St Louis Criteria

Explanation:

For a diagnosis of schizophrenia, DSM-5 requires two or more the following to be present: delusions, hallucinations, disorganised speech, grossly disorganised or catatonic behaviour, negative symptoms. Continuous signs of disturbance must last for at least 6 months, including at least one month of active symptoms.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 185.

The correct answer is: DSM-5

Question **54**

Not answered

Marked out of 1.00

Flag question

Which of the following disorders is believed by some to represent a point on a continuum of 'unitary psychosis' lying quantitatively between schizophrenic and mood symptoms?

Select one:

- Organic mood disorders
- Schizoaffective disorder
- Bipolar affective disorder
- Psychotic depression
- Post schizophrenic depression

Explanation:

Schizoaffective disorder has features of both affective disorder and schizophrenia which are present in approximately equal proportion. Its nosological status is uncertain, since some believe it to be a variant of schizophrenia; others, bipolar disorder; and some believe it represents a point on a continuum of ‘unitary psychosis’, lying between schizophrenia and mood disorders.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 228.

The correct answer is: Schizoaffective disorder

Question **55**

Not answered

Marked out of 1.00

Flag question

Which of the following is the best test to differentiate delirium from dementia?

Select one:

- Altered consciousness
- Poor recall
- Neurological localising signs
- Presence of visual hallucinations

- Presence of delusions

Explanation:

With delirium, attention and level of consciousness are reduced and fluctuating; with dementia these domains typically remain intact until the advanced stages of dementia.

Ref: Fong TG et al. The interface of delirium and dementia in older persons. The Lancet. Neurology. 2015 Aug;14(8):823.

The correct answer is: Altered consciousness

Question **56**

Not answered

Marked out of 1.00

Flag question

According to ICD 11 what is the minimum duration required for a diagnosis of hypomania?

Select one:

- 2 weeks
- Several days
- 2 months
- Several weeks
- 1 week

Explanation:

According to ICD-11, a hypomanic episode is a persistent mood state lasting at least several days characterised by mild elevation of mood or increased irritability and increased activity or a subjective experience of increased energy, accompanied by other characteristic symptoms such as rapid speech, rapid or racing thoughts, increased self-esteem, an increase in sexual drive or sociability, decreased need for sleep, distractibility, or impulsive or reckless behaviour.

Unlike its prior editions such as ICD-10, ICD-11 does not provide a specific duration of days to differentiate hypomania and mania.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Several days

Question **57**

Not answered

Marked out of 1.00

Flag question

Autism spectrum disorder typically first becomes evident at what age?

Select one:

- < 2 years old
- 3-5 years old
- 6-10 years old
- 12-16 years old
- > 18 years old

Explanation:

Autism spectrum disorder (ASD) is typically evident during the second year of life, and in severe cases, a lack of developmentally appropriate interest in social interactions may be noted even in the first year. However, in milder cases, core impairments in ASD may not be identified for several more years. ASD in children with normal intellectual function and mild impairment in language function may not be identified until middle childhood when both academic and social demands increase.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 122.

The correct answer is: < 2 years old

Question **58**

Not answered

Marked out of 1.00

Flag question

The term simple schizophrenia was added to the categories of existing schizophrenias by

Select one:

- Bleuler
- Hubbard
- Hecker
- Angst
- Liddle

Explanation:

In 1896, Emil Kraepelin described and separated the two major forms of insanity on the basis of different symptoms, course, and outcome. The first, 'manic-depressive insanity', had a relapsing and remitting course, with full recovery after each episode. The second grouped together catatonia, hebephrenia, and the paranoid psychoses under the term 'dementia praecox', which had a progressive, deteriorating course. Eugen Bleuler added 'simple schizophrenia' to Kraepelin's subtypes and did not consider hallucinations, delusions, and catatonic symptoms to be necessary for the diagnosis. He believed the disorder to be due to a 'loosening of associations' between psychic functions, with fundamental symptoms being thought disorder, blunting/incongruity of affect, autism, and ambivalence.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 182.

The correct answer is: Bleuler

Question **59**

Not answered

Marked out of 1.00

Flag question

A 23-year-old man is hospitalized for a fever of unknown origin and persistent cough. He also complains of being increasingly forgetful and having difficulties driving through familiar routes. Physical examination reveals several needle marks in his forearms. The most likely diagnosis is

Select one:

- HIV-associated neurocognitive disorder
- Prion disease
- Cocaine-induced brain damage
- Endocarditis
- Alcoholic dementia

Explanation:

Neurocognitive impairment is a common problem impacting the quality of life and medication adherence in people with HIV. HIV-associated neurocognitive dysfunction (HAND) refers to a spectrum of neurocognitive impairments that consist of asymptomatic neurocognitive impairment, mild neurocognitive disorder, and HIV-associated dementia. Symptoms of mild neurocognitive disorder include cognitive slowing and memory deficits, as well as motor slowing and subtle incoordination. Clinical features of HIV-associated dementia are classified as cognitive (subcortical dementia, focal cognitive deficits, amnesia, mutism), motor (movement disorders e.g. tremor, ataxia, choreo-athetosis, spasticity, myoclonus), and affective (depression, apathy, agitation, disinhibition, mania).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 149.

Alford K, Vera JH. Cognitive impairment in people living with HIV in the ART era: a review. British medical bulletin. 2018 Sep 1;127(1):55-68.

The correct answer is: HIV-associated neurocognitive disorder

Question **60**

Not answered

Marked out of 1.00

Flag question

A 16-year-old boy who has used cannabis for the first time complains that his legs are being pulled and moved. Which of the following is most likely?

Select one:

- Acute cannabis effect
- Dyskinesia
- Complex seizure
- Schizophrenia
- Delusional disorder

Kinaesthetic hallucinations and passivity can occur under the acute intoxication-related effects of cannabis.

The correct answer is: Acute cannabis effect

Question **61**

Not answered

Marked out of 1.00

Flag question

Which one among the following is NOT a core diagnostic feature of ADHD?

Select one:

- Language dysfunction
- Inattention
- Hyperactivity
- Impulsivity
- None of the listed features

Explanation:

ADHD is characterised by the three core symptoms of inattention, hyperactivity, and impulsiveness.

Hyperactivity-impulsivity symptoms include: fidgeting, moving, getting up and running about, talking excessively; unable to play quietly; continually interrupting.

Inattention symptoms include: being easily distracted, poor attention span and not able to sustain attention; poor task completion; inability to organize, and making mistakes with tasks that require attention; the child is forgetful, and often loses items of daily use.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

The correct answer is: Language dysfunction

Question **62**

Not answered

Marked out of 1.00

Flag question

A 34-year-old woman appears flamboyant and dramatic in her behaviour, drawing excessive attention to herself. Which of the following is very likely?

Select one:

- Perfectionistic behaviour
- Considering relationships to be more intimate than they are
- Rapid mood swings
- Anxiety in social situations
- Preoccupation with rejection

Explanation:

Clinical features of patients with histrionic personality disorder include: being overly dramatic and theatrical; considering relationships to be more intimate than they are; using physical appearance to draw attention to onself; speaking in a way that is excessively impressionistic and lacking in detail; having shallow and shifting emotions; being inappropriately sexually provocative; feeling uncomfortable when not the centre of attention; being suggestible and easily influenced.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 564-565.

The correct answer is: Considering relationships to be more intimate than they are

Question **63**

Not answered

Marked out of 1.00

Flag question

A man who recently witnessed his colleague's death in an unfortunate motor vehicle collision presents with weakness of both legs. He is not able to balance himself when asked to stand and needs help to support him. On physical examination, no consistent neurological signs are noted. The most likely diagnosis is

Select one:

- Somatoform disorder
- Somatisation disorder
- Conversion disorder
- Malingering
- PTSD

Explanation:

In dissociative (conversion) disorders, there is a loss or disturbance of normal motor, sensory, or cognitive functions, which initially appears to have a neurological or other physical cause but is later attributed to a psychological cause. Classic conversion syndromes represent neurologic syndromes such as paralysis, seizures, or blindness. The motor symptoms of conversion disorder include abnormal movements, gait disturbance, weakness, and paralysis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 868.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 454-455.

The correct answer is: Conversion disorder

Question **64**

Not answered

Marked out of 1.00

Flag question

Which of the following is true regarding delusional disorder according to ICD-11 diagnostic criteria?

Select one:

- No hallucinations can be present
- Only a single delusion can be present
- Delusional thought should be present for at least 3 months
- Delusions must be incongruent to mood
- A preceding stressor should be identifiable

Explanation:

Delusional disorder is characterised by the development of a delusion or set of related delusions, typically persisting for at least 3 months and often much longer, in the absence of a depressive, manic, or mixed mood episode. The delusions are variable in content across individuals, but typically stable within individuals, although they may evolve over time. Other characteristic symptoms of schizophrenia (i.e. clear and persistent hallucinations, negative symptoms, disorganised thinking, or experiences of influence, passivity, or control) are not present, although various forms of perceptual disturbances (e.g. hallucinations, illusions, misidentification of persons) thematically related to the delusion are still consistent with the diagnosis.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Delusional thought should be present for at least 3 months

Question **65**

Not answered

Marked out of 1.00

Flag question

Which of the following is not a dimensional descriptor for schizophrenia in ICD-11?

Select one:

- Depressive mood symptoms
- Manic mood symptoms
- Anxiety symptoms
- Psychomotor symptoms
- Cognitive symptoms

Explanation:

In ICD-11, a set of dimensional descriptors for schizophrenia (and other primary psychotic disorders) has been introduced. These include: **positive symptoms** (delusions, hallucinations, disorganised thinking and behaviour, experiences of passivity and control); **negative symptoms** (constricted, blunted or flat affect, alogia or paucity of speech, avolition, anhedonia); **depressive mood symptoms**; **manic mood symptoms**; **psychomotor symptoms** (psychomotor agitation, psychomotor retardation, catatonic symptoms); and **cognitive symptoms** (particularly deficits in speed of processing, attention/concentration, orientation, judgement, abstraction, verbal or visual learning, and working memory).

Ref: Reed GM et al. Innovations and changes in the ICD-11 classification of mental, behavioural and neurodevelopmental disorders. World Psychiatry. 2019 Feb;18(1):3-19.

The correct answer is: Anxiety symptoms

Question **66**

Not answered

Marked out of 1.00

Flag question

A 40-year-old patient with a history of long-standing problems with hearing voices and feeling under threat from the government now presents with waxy flexibility and resistance to all instructions. He repeats whatever is said during the interview in a stilted voice but does not speak spontaneously. The most likely diagnosis is which of the following?

Select one:

- Malingering
- Drug-induced psychosis
- Neuroleptic malignant syndrome
- Catatonia associated with another mental disorder
- Selective mutism

Explanation:

Catatonia associated with another mental disorder is a syndrome of primarily psychomotor disturbance that is characterised by the simultaneous occurrence of several symptoms such as stupor; catalepsy; waxy flexibility; mutism; negativism; posturing; mannerisms; stereotypies; psychomotor agitation; grimacing; echolalia and echopraxia. Mental disorders associated with catatonia include mood disorders, schizophrenia or other primary psychotic disorders, and neurodevelopmental disorders, especially autism spectrum disorder.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Catatonia associated with another mental disorder

Question **67**

Not answered

Marked out of 1.00

Flag question

A patient with schizophrenia experienced his most recent relapse 8 months ago. He currently has low mood, anhedonia, flat affect, apathy and suspiciousness over religious people. For the last 2 weeks he has not eaten well, currently avoids eye contact and appears tearful. Which of the following diagnosis is appropriate?

Select one:

- First-episode schizophrenia
- Treatment resistant schizophrenia

- Mixed bipolar episode
- Depressive mood symptoms in schizophrenia
- Other psychotic disorder

Explanation:

Depression and depressive features are relatively common in schizophrenia. Some evidence suggests that negative cognitive appraisals of shame and loss and more prominent in post-psychotic depression as compared with major depressive disorders, whereas in the acute phases of psychosis, depression may be more related to the experience of positive symptoms themselves: perceived threat from powerful persecutors and/or malevolent hallucinations.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Depressive mood symptoms in schizophrenia

Question **68**

Not answered

Marked out of 1.00

Flag question

The symptoms of adjustment disorder resolve within a period of

Select one:

- 3 months of termination of the stressor
- 1 week of termination of the stressor
- 2 weeks of termination of the stressor
- 6 months of termination of the stressor
- 1 month of termination of the stressor

Explanation:

Symptoms of an adjustment disorder should not persist for longer than 6 months after the stressor (or its consequences) is removed.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 398.

The correct answer is: 6 months of termination of the stressor

Question **69**

Not answered

Marked out of 1.00

Flag question

Characteristic features of delirium tremens include all except

Select one:

- Diarrhoea
- Fluctuant blood pressure and pulse
- Hallucinations
- Clouded consciousness
- Agitation

Explanation:

Characteristics of delirium tremens include clouding of consciousness; disorientation; amnesia for recent events; marked psychomotor agitation; visual, auditory, and tactile hallucinations (characteristically of diminutive people or animals – 'Lilliputian' hallucinations); and marked fluctuations in severity hour by hour. In severe cases there can be heavy sweating, fear, paranoid delusions, agitation, suggestibility, raised temperature, and/or sudden cardiovascular collapse.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 590.

The correct answer is: Diarrhoea

Question **70**

Not answered

Marked out of 1.00

Flag question

A 42-year-old intellectually disabled man lives in a sheltered accommodation. He attends day centre twice a week and gets regular visitors. He cannot write or read but can speak using restricted phrases. He needs supervision for daily activities and cannot be trained easily. His social worker helps him with finances. What level of intellectual disability does he have?

Select one:

- Below average
- Moderate
- Severe
- Mild
- Profound

Explanation:

Level of intellectual disability	Level of functioning
Mild	Often struggle with academic skills and complex language comprehension Most are capable of basic self-care and domestic and practical activities Most are able to live relatively independently Capable of finding employment as adults, but may need appropriate support
Moderate	Capacity for academic skills and language comprehension varies, usually limited to basic skills Some can manage basic self-care and domestic and practical activities Most need significant ongoing support to live independently and maintain employment as adults
Severe	Very limited capacity for language and ability to gain academic skills Often accompanied by motor impairments Daily support in a supervised environment is usually required Some may develop basic self-care skills with intensive support and training
Profound	Very limited communication ability Potential to gain academic skills is limited to basic concrete skills Motor and sensory impairments usually present Daily support in a supervised care environment usually required

Adapted from Williams et al. 2018

SPMM COURSE

Ref: Williams F et al. Alcohol use disorders in people with intellectual disability. BJPsych Advances. 2018 Jul;24(4):264-72.

The correct answer is: Severe

Question **71**

Not answered

Marked out of 1.00

Flag question

A 24-year-old lady suddenly develops bilateral blindness. She has patchy tunnel-like vision on field testing. She appears not to be bothered by the problem. This is described as

Select one:

- La belle indifference
- Anosognosia
- Loss of insight
- Malingering
- Anton syndrome

Explanation:

(La) belle indifference is a surprising lack of concern for, or denial of, apparently severe functional disability. It is part of classical descriptions of hysteria and continues to be associated with operational descriptions of conversion disorder. It is also seen in medical illnesses (e.g. following a stroke) and is a rare and non-specific symptom of no diagnostic value.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 103.

The correct answer is: La belle indifference

Question **72**

Not answered

Marked out of 1.00

Flag question

What is the minimum symptom duration required to make an ICD-11 diagnosis of schizophrenia?

Select one:

- 6 months
- 3 months
- 1 month
- 1 week
- 1 day

Explanation:

For an ICD-11 diagnosis of schizophrenia, a symptom duration of at least 1 month is generally required. With DSM-5, psychotic symptoms (criterion A) needs to be present for a month while the total duration of disorder itself should be for at least 6 months.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: 1 month

Question **73**

Not answered

Marked out of 1.00

Mr. X has been treated recently for hypomania. He also has a previous history of depression. Which one among the following is his correct diagnosis?

Select one:

Flag question

Recurrent depressive disorder

- Bipolar disorder type 1
- Bipolar disorder type 2
- Rapid cycling disorder
- Mixed affective state

Explanation:

Diagnosis of bipolar disorder:

- In **DSM-5** and **ICD-11** bipolar disorders are **subdivided** into **bipolar I** and **bipolar II** disorder
- **Bipolar I** requires **at least one past or present manic or mixed episode**; **depressive episodes** may occur but **are not obligatory**
- **Bipolar II** requires **at least one hypomanic and one depressive episode**

Mixed states are cases where manic and depressive symptoms occur simultaneously. Rapid Cycling: When at least four episodes of bipolar disorder occur within a period of one year, the condition is described as 'rapid cycling'.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 328.

The correct answer is: Bipolar disorder type 2

Question **74**

Not answered

Marked out of 1.00

Flag question

Which of the following is a neurological cause of catatonia?

Select one:

- Hypercalcaemia
- Adverse drug effect of a neuroleptic medication
- Encephalitis
- Hepatic encephalopathy
- Adverse drug effect of phencyclidine

Explanation:

The most common psychiatric disorders causing catatonia include major depression and schizophrenia. Catatonia can also appear as an adverse drug effect of a neuroleptic medication or phencyclidine.

Neurological causes of catatonia include post-encephalitic states, parkinsonism, seizure disorder, bilateral globus pallidus disease, lesions of the thalamus or parietal lobes, frontal lobe disease, and general paresis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30.

The correct answer is: Encephalitis

Question **75**

Not answered

Which one of the following disorders is characterised by normal language development?

Marked out of 1.00

Flag question

Select one:

- Asperger syndrome
- Rett syndrome
- High functioning autism
- Autism
- Childhood disintegrative disorder

Explanation:

Asperger syndrome is characterised by severe persistent impairment in reciprocal social interactions, repetitive behaviour patterns, and restricted interests. IQ and language are normal or, in some cases, superior. Mild motor clumsiness and a family history of autism may be present.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 820.

The correct answer is: Asperger syndrome

Question **76**

Not answered

Marked out of 1.00

Flag question

A 21-year-old woman often falls asleep during the day. She has fallen asleep while speaking to her manager twice, causing problems at work. Her night time sleep is undisturbed, and other spheres of functioning are satisfactory. Physical health is unremarkable except for a history of an episode of fall 2 weeks ago when she was on a train laughing heartily with her boyfriend. She did not sustain any head injury then. Which of the following is the most appropriate diagnosis for this case?

Select one:

- narcolepsy
- hypersomnia not otherwise specified
- primary hypersomnia
- circadian rhythm sleep disorder
- no illness

Explanation:

Classic symptoms of narcolepsy include: excessive daytime sleepiness; cataplexy (sudden loss of muscle tone during wakefulness that is evoked by strong, mainly positive, emotions); sleep paralysis (sometimes up to 10 minutes long); and hypnagogic hallucinations (vivid hallucinations on falling asleep) or hypnopompic hallucinations (hallucinations when the individual is waking up).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 450.

Kornum BR et al. Narcolepsy. Nature reviews Disease primers. 2017 Feb 9;3(1):1-9.

The correct answer is: narcolepsy

Question **77**

Not answered

Marked out of

The year agreed by member states of the World Health Organisation (WHO) to bring ICD-11 into practice is

1.00

Flag question

Select one:

- 1992
- 2022
- 2019
- 1948
- 1994

Explanation:

In May 2019, member states of the World Health Assembly adopted ICD-11 to come into practice as of **January 1, 2022**.

Ref: https://www.cihi.ca/en/icd-11-frequently-asked-questions

The correct answer is: 2022

Question **78**

Not answered

Marked out of 1.00

Flag question

The symptom qualifiers ‘positive symptoms’, ‘negative symptoms’, ‘depressive mood symptoms’, manic mood symptoms’, psychomotor symptoms’, and ‘cognitive symptoms’ may be used for any of the following ICD-11 diagnoses EXCEPT

Select one:

- Schizophrenia
- Schizoaffective disorder
- Bipolar disorder
- Delusional disorder
- Schizotypal disorder

Explanation:

The dimensional symptom specifiers listed here may be used for any diagnosis from ICD-11’s ‘Schizophrenia or Other Primary Psychotic Disorders’ grouping. This grouping includes schizophrenia, schizoaffective disorder, schizotypal disorder, acute and transient psychotic disorder, and delusional disorder.

Ref: Gaebel W, Stricker J, Kerst A. Changes from ICD-10 to ICD-11 and future directions in psychiatric classification. Dialogues in clinical neuroscience. 2020.

International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Bipolar disorder

Question **79**

Not answered

Marked out of 1.00

According to DSM-5, caffeine intoxication is defined as recent consumption of caffeine usually in excess of

Select one:

- 100 mg

Flag question

- 50 mg
- 250 mg
- 200 mg
- 150 mg

Explanation:

DSM-5 describes caffeine intoxication as recent consumption of caffeine, usually in excess of 250mg. Common symptoms associated with caffeine intoxication include anxiety, psychomotor agitation, restlessness, irritability, and psycho-physiological complaints such as muscle twitching, flushed face, nausea, diuresis, gastrointestinal distress, excessive perspiration, tingling in the fingers and toes, and insomnia.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 312.

The correct answer is: 250 mg

Question **80**

Not answered

Marked out of 1.00

Flag question

Which of the following clinical phenomenon is associated with the name Angst?

Select one:

- Folie a deux
- Cycloid psychosis
- Mixed affective states
- Bipolarity of mood disorders
- Unitary psychosis

Explanation:

Jules Angst, whose early work was central to the move to a dichotomous view of the mood disorders in the 1960s, became an advocate for the bipolar spectrum concept (‘bipolarity’) when, in later studies, he found many intermediate forms between the original bipolar and unipolar ideal types, with mixed states occurring in up to 50% of all depressive conditions. These findings brought into question the idea of ‘polarity’ as a useful distinction.

Ref: Angst J et al. Bipolar spectrum in major depressive disorders. European archives of psychiatry and clinical neuroscience. 2018 Dec;268(8):741-8.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 326.

The correct answer is: Bipolarity of mood disorders

Question **81**

Not answered

Marked out of 1.00

Flag question

A man who has always had an anxious disposition has been feeling increasingly on edge. He has symptoms of breathlessness, chest pain and nausea that have been worsening over the last 6 months. What is the most likely diagnosis?

Select one:

- Adjustment disorder

- Generalised anxiety disorder
- Mixed anxiety and depressive disorder
- Agoraphobia with panic disorder
- Da Costa syndrome

Explanation:

Generalised anxiety disorder is characterised by marked symptoms of anxiety that persist for at least several months, for more days than not, manifested by either general apprehension (i.e. 'free-floating anxiety') or excessive worry focused on multiple everyday events, most often concerning family, health, finances, and school or work, together with additional symptoms such as muscular tension or motor restlessness, sympathetic autonomic overactivity, subjective experience of nervousness, difficulty maintaining concentration, irritability, or sleep disturbance.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Generalised anxiety disorder

Question **82**

Not answered

Marked out of 1.00

Flag question

Which one of the following symptoms is most frequently noted in schizophrenic patients?

Select one:

- Auditory hallucinations
- Ideas of reference
- Suspiciousness
- Flatness of affect
- Lack of insight

Explanation:

Impairment of insight is considered as the hallmark of schizophrenia. Insight is impaired in patients with schizophrenia, and this is almost a defining feature. According to the International Pilot Study on Schizophrenia, a substantial proportion of patients with schizophrenia (up to 80%) demonstrate poor insight into the illness, irrespective of the cultural variation of patients.

Ref: Joseph B et al. Insight in schizophrenia: relationship to positive, negative and neurocognitive dimensions. Indian journal of psychological medicine. 2015 Jan;37(1):5-11.

The correct answer is: Lack of insight

Question **83**

Not answered

Marked out of 1.00

Flag question

A 32-year-old former nurse often presents to A and E with a plethora of complaints. She has undergone many surgeries in the past. During one such admission, when confronted, she admits to injecting faeces into her blood in order to produce a septic blood culture for no apparent reason. What is the most likely diagnosis?

Select one:

- Malingering
- Dissociative disorder

- Munchausen syndrome
- Conversion disorder
- Somatisation disorder

Explanation:

In Munchausen syndrome (factitious disorder), patients intentionally falsify their symptoms and past history and fabricate signs of physical or mental disorder, with the primary aim of obtaining medical attention and treatment.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 876.

The correct answer is: Munchausen syndrome

Question **84**

Not answered

Marked out of 1.00

Flag question

A man brings his mother to stay with his family permanently, following her retirement. Soon after this, his wife becomes aphonic (mute). Physical examination reveals no neurological explanation. The most likely diagnosis is

Select one:

- Somatisation disorder
- Dissociative neurological symptom disorder
- Somatoform disorder
- Malingering
- Hypochondriasis

Explanation:

Dissociative neurological symptom disorder (DNSD) with speech disturbance is characterised by symptoms such as difficulty with speaking (dysphonia), loss of the ability to speak (aphonia) or difficult or unclear articulation of speech (dysarthria) that are not consistent with a recognised disease of the nervous system, a neurodevelopmental or neurocognitive disorder, other mental disorder, or another medical condition and do not occur exclusively during another dissociative disorder.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Dissociative neurological symptom disorder

Question **85**

Not answered

Marked out of 1.00

Flag question

Identify the memory feature more commonly seen in dissociative disorders but not in organic amnesia.

Select one:

- Intact procedural memory
- Impaired episodic memory
- Intact semantic memory
- Loss of personal identity

- All of the listed features

Explanation:

Memory loss seen in dissociative (conversion) disorders is most often for recent events, is not attributable to an organic mental disorder, and is too severe to attribute to ordinary forgetfulness. The amnesia is usually patchy and selective – true global amnesia is rare. The patient may have variable amnesia for personal information, including personal identity, which is not seen in organic amnesia. Despite the amnesia, the patient is able to function and retains procedural memory.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 869.

The correct answer is: Loss of personal identity

Question **86**

Not answered

Marked out of 1.00

Flag question

Which one of the following is not a feature of Wernicke's encephalopathy?

Select one:

- Confusion
- Memory disturbance
- Ataxia
- Nystagmus
- Hyperthermia

Explanation:

Wernicke encephalopathy (WE) is an acute condition characterised by the classic triad of confusion, ataxia, and ophthalmoplegia/nystagmus. However, this triad only seen in 10% of cases. There is confusion in 80% of cases. WE occurs secondary to thiamine (vitamin B1) deficiency.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 606.

The correct answer is: Hyperthermia

Question **87**

Not answered

Marked out of 1.00

Flag question

A 34-year-old woman becomes very tearful when her husband has to travel for work as she thinks she is unable to function without him. This is a feature of

Select one:

- Histrionic personality disorder
- Borderline personality disorder
- Obsessive-compulsive personality disorder
- Avoidant personality disorder
- Dependent personality disorder

Explanation:

Clinical features of patients with dependent personality disorder include difficulty expressing disagreement; need for others to assume responsibility for most areas of their daily life; lack of self-confidence; feeling unable to function independently; volunteering to do unpleasant things for nurturance and support; difficulty making every-day decisions without excessive amounts of advice; urgently seeking another close relationship when one ends; fear of being left to take care of themselves.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 565-566.

The correct answer is: Dependent personality disorder

Question **88**

Not answered

Marked out of 1.00

Flag question

When depression occurs in the context of acute psychosis in schizophrenia, the depression is most likely to be related to

Select one:

- Shame associated with the diagnosis of schizophrenia
- Loss of day-to-day functionality due to schizophrenia
- Loss of employment due to schizophrenia
- Perceived threats and/or malevolent hallucinations
- None of the listed options

Explanation:

Depression and depressive features are relatively common in schizophrenia. Some evidence suggests that negative cognitive appraisals of shame and loss and more prominent in post-psychotic depression as compared with major depressive disorders, whereas in the acute phases of psychosis, depression may be more related to the experience of positive symptoms themselves: perceived threat from powerful persecutors and/or malevolent hallucinations.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Perceived threats and/or malevolent hallucinations

Question **89**

Not answered

Marked out of 1.00

Flag question

Choose one best option regarding the distribution of schizophrenia

Select one:

- The incidence of schizophrenia is higher in developing countries
- Schizophrenia is more common among women than men
- Higher parental income during childhood is associated with lower risk of schizophrenia
- 2% of population has schizophrenia at any given time
- The prevalence of schizophrenia is higher in developing countries

Explanation:

Social drift is associated with a diagnosis of schizophrenia. A Danish national cohort study reported

that parental income level and income mobility during childhood are linked with schizophrenia risk.

Ref: Hakulinen C, Webb RT, Pedersen CB, Agerbo E, Mok PL. Association between parental income during childhood and risk of schizophrenia later in life. JAMA psychiatry. 2020 Jan 1;77(1):17-24.

The correct answer is: Higher parental income during childhood is associated with lower risk of schizophrenia

Question **90**

Not answered

Marked out of 1.00

Flag question

Different types of panic attacks include all except

Select one:

- Nocturnal panic attacks
- Situationally predisposed attacks
- Unilateral panic attacks
- Situational panic attacks
- Out of blue panic attacks

Panic attacks cannot be unilateral. They are associated with a generalised autonomic activity and not directly related to cortical seizures that could be unilateral.

The correct answer is: Unilateral panic attacks

Question **91**

Not answered

Marked out of 1.00

Flag question

Who first described bulimia nervosa?

Select one:

- Crow
- Liddle
- Russell
- Griesinger
- Klerman and Weissman

Explanation:

Bulimia nervosa was first described in 1979 by British psychiatrist Gerald Russell as a ‘chronic phase of anorexia nervosa’ in which patients overeat and then use compensatory mechanisms such as self-induced vomiting, laxatives, or prolonged periods of starvation.

Griesinger described unitary psychosis in 1868. Crow published his two syndrome (Type 1 and 2) hypothesis of schizophrenia in 1980. Liddle described the three-syndrome model of schizophrenia in 1987. Klerman and Weissman introduced interpersonal psychotherapy.

Ref: Castillo M, Weiselberg E. Bulimia nervosa/purging disorder. Current problems in pediatric and adolescent health care. 2017 Apr 1;47(4):85-94.

The correct answer is: Russell

Question **92**

Not answered

Marked out of 1.00

Flag question

Choose the one feature which is not defined under Edwards and Gross criteria (1976) for alcohol dependence syndrome.

Select one:

- repeated withdrawals
- reinstatement after abstinence
- narrowed repertoire
- decreased tolerance
- subjective awareness of compulsion to drink

Explanation:

Original Edwards & Gross criteria for alcohol dependence (1976):

1. Narrowing of the drinking repertoire
2. Salience/primacy of alcohol-seeking behaviour
3. Increased tolerance
4. Repeated withdrawal symptoms
5. Relief or avoidance of withdrawal symptoms by further drinking
6. Subjective awareness of compulsion to drink
7. Reinstatement after abstinence

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: decreased tolerance

Question **93**

Not answered

Marked out of 1.00

Flag question

Which one of the following is not known to be associated with an occurrence of psychotic features during the episode?

Select one:

- Bipolar affective disorder - current episode mania
- Psychotic depression
- Acute and transient psychotic disorder
- Hypomanic episode
- Schizoaffective disorder

Explanation:

Psychotic features are not seen during a hypomanic episode. The clinical features of hypomania include elated mood, increased energy levels, excitability, lack of sleep, over-optimistic ideation and expansive mood. By definition, hypomania is not severe enough to interfere with social or

occupational functioning, require admission to hospital, or include psychotic features.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 322.

The correct answer is: Hypomanic episode

Question **94**

Not answered

Marked out of 1.00

Flag question

Following the death of her 85 year old mother 4 months ago, a 49-year-old receptionist has become increasingly convinced that the prime minister is in love with her and that they have an ongoing affair. She has clear thoughts and no hallucinations. She is doing well at work and has no issues with her husband. Which of the following is the most likely diagnosis?

Select one:

- delusional disorder
- acute reactive psychosis
- paranoid personality disorder
- prodromal schizophrenia
- schizophreniform disorder

Explanation:

Delusions without hallucinations and thought disorder, along with preserved family and work functioning, are suggestive of a delusional disorder. Delusional disorder is an uncommon condition in which patients present with circumscribed symptoms of non-bizarre delusions (DSM-5 allows 'with bizarre content'; ICD-11 does not specify), but with absence of prominent hallucinations and no thought disorder, mood disorder, or significant flattening of affect. DSM-5 subtypes include erotomanic, grandiose, jealous, persecutory, and somatic. ICD-11 states that delusions typically persist for at least 3 months and often much longer.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 230.

The correct answer is: delusional disorder

Question **95**

Not answered

Marked out of 1.00

Flag question

A child is suspected to have Asperger syndrome. Which of the following is not consistent with the diagnosis of Asperger syndrome?

Select one:

- Restricted interests
- Clumsy motor coordination
- Abnormalities in reciprocal social interaction
- Stereotyped motor activities
- Poor speech development till age 3

Explanation:

Asperger syndrome is characterised by severe persistent impairment in reciprocal social interactions, repetitive behaviour patterns, and restricted interests. IQ and language are normal or, in some cases,

superior. Mild motor clumsiness and a family history of autism may be present.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 820.

The correct answer is: Poor speech development till age 3

Question **96**

Not answered

Marked out of 1.00

Flag question

Which of the following statements regarding PTSD is true?

Select one:

- Specific EEG changes occur in chronic PTSD
- Previous history of neurotic illness precludes the development of PTSD features
- PTSD is associated with hypersomnia
- PTSD symptoms usually occur within 6 months of the trauma
- Autonomic hyperarousal symptoms are usually rare

Explanation:

Symptoms of PTSD usually arise within 6 months of the traumatic event. There is delayed onset in approximately 10% of cases. The essential features of PTSD are autonomic hyperarousal (e.g. exaggerated startle response), re-experiencing of aspects of the traumatic event, and active avoidance of cues that remind the person of the trauma. Anhedonia may also be seen. Insomnia is common. Vulnerability factors for PTSD include low self-esteem/neurotic traits and previous (or family) history of psychiatric problems (especially mood/anxiety disorders).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 402-403.

The correct answer is: PTSD symptoms usually occur within 6 months of the trauma

Question **97**

Not answered

Marked out of 1.00

Flag question

Mr.X is a 27-year-old gentleman with a long standing diagnosis of schizophrenic illness. He has both positive and negative symptoms of schizophrenia. Which among the following is classified as a negative symptom of schizophrenia?

Select one:

- Hallucinations
- Bizarre behaviour
- Affective flattening
- Delusions
- Formal thought disorder

Explanation:

Negative symptoms of schizophrenia are aspects of normal mental and physical functioning **diminished or lost** with illness including:

1. **Flattened affect** (limited affective responsiveness)

2. **Alogia** (poverty of speech)
3. **Anhedonia** (inability to experience pleasure)
4. **Social withdrawal** (lack of desire to form relationships)
5. **Avolition** (lack of motivation)
6. **Lethargy**
7. **Attentional impairment**

Primary negative symptoms = associated with underlying pathophysiology

Secondary negative symptoms = attributable to other factors like residual positive symptoms (e.g. paranoia causing social withdrawal), medication side effects, and overlap with depression

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021.

The correct answer is: Affective flattening

Question **98**

Not answered

Marked out of 1.00

Flag question

What is the time duration before which most cases with acute uncomplicated delirium recover?

Select one:

- 4 weeks
- 6 months
- 3 days
- 1 day
- 1 year

Most cases of delirium recover in 4 weeks; in chronic lung disease, SABE (subacute bacterial endocarditis) and carcinoma up to 6 months long delirium may be seen

The correct answer is: 4 weeks

Question **99**

Not answered

Marked out of 1.00

Flag question

A woman comes to your outpatient unit with a diagnosis of OCD made by her GP. Which of the following symptoms is most likely in this patient?

Select one:

- Counting
- Hoarding
- Checking
- Rearranging items on the table
- Cleaning

Explanation:

Checking is one of the most common features of OCD. It is more common than other symptoms listed in this question. A possible sex-bias in adolescent community samples with boys reporting more checking behaviours, while girls reporting more cleaning has been noted in some studies.

Ref: Fullana, M.A., et al., 2009. Obsessions and compulsions in the community: prevalence, interference, help-seeking, developmental stability, and co-occurring psychiatric conditions. *American Journal of Psychiatry*, *166*(3), pp.329-336.

Barzilay R, et al. Obsessive-compulsive symptomatology in community youth: typical development or a red flag for psychopathology?. Journal of the American Academy of Child & Adolescent Psychiatry. 2019 Feb 1;58(2):277-86.

The correct answer is: Checking

Question **100**

Not answered

Marked out of 1.00

Flag question

In PTSD, which of the following is associated with a poor prognosis?

Select one:

- Vivid memory of the trauma
- Reduced threat intensity of recalled trauma
- Florid emotional response at the time of trauma
- Sense of numbing
- Unemployment at the time of trauma

Explanation:

Numbing in PTSD has been associated with several distress-related outcomes, including chronicity of distress, impaired interpersonal functioning, and lower overall quality of life.

Ref: Hassija CM et al. Numbing and dysphoria symptoms of posttraumatic stress disorder among Iraq and Afghanistan war veterans: A review of findings and implications for treatment. Behavior modification. 2012 Nov;36(6):834-56.

The correct answer is: Sense of numbing

Question **101**

Not answered

Marked out of 1.00

Flag question

A 35-year-old man has low mood, anhedonia, loss of hope for the future, loss of appetite, and poor sleep. He has no other symptoms of depression and is able to function in some areas of daily life, but struggles considerably when undertaking most activities compared to his state several months ago. What is the severity of his depression?

Select one:

- Mild
- Dysthymia
- Severe
- No depression
- Moderate

Explanation:

In ICD-11, ‘mild’ is when none of the symptoms of a depressive episode are intense, and there is some, but not considerable, difficulty in continuing normal activities, and no delusions or hallucinations. ‘Moderate’ is when several symptoms of a depressive episode are present to a marked degree or a large number of depressive symptoms of lesser severity are present overall and there is considerable difficulty in continuing with normal activities, but the individual is still able to function in at least some areas. ‘Severe’ is when many or most symptoms of a depressive episode are present to a marked degree or a smaller number of symptoms are present and manifest to an intense degree and the individual is unable to function, expect to a very limited degree.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 250.

The correct answer is: Moderate

Question **102**

Not answered

Marked out of 1.00

Flag question

A 49-year-old man has been highly alcohol dependent. Due to various physical problems he was strongly advised to abstain. Unfortunately since his last episode of intoxication he has developed recurrent voices that frighten him. In spite of 2 months of abstinence these hallucinations continue, and he now presents with persecutory delusions. Which of the following diagnosis best suits his presentation?

Select one:

- Depressive psychosis
- Delirium tremens
- Schizophrenia
- Alcohol-induced psychotic disorder with hallucinations
- Delusional disorder

Explanation:

Substance-induced psychotic illness is a rare complication of prolonged heavy alcohol use. This has been called alcoholic hallucinosis, but in ICD-11 it is designated as ‘alcohol-induced psychotic disorder with hallucinations’. It is characterised by experiencing hallucinations (usually auditory) while sober. It usually starts with elemental hallucinations (e.g. bangs or murmurings), and with continued alcohol use they become formed voices, usually derogatory. Unlike delirium tremens, these patients have a clear sensorium. In 95% of patients, symptoms rapidly resolve on ceasing alcohol consumption, but rapidly recur on restarting drinking; in 5%, symptoms persist with abstinence.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 602.

The correct answer is: Alcohol-induced psychotic disorder with hallucinations

Question **103**

Not answered

Marked out of 1.00

Flag question

Which of the following is true with respect to narcissistic personality disorder?

Select one:

- There is a markedly higher degree of egocentricity than in the other personality disorders
- It is included in both ICD-10 and ICD-11
- It is associated with more malevolent feelings towards others than in antisocial personality

disorder

- It is a common diagnosis in Europe
- There may be a preoccupation with fantasies of success, power, and brilliance

Explanation:

Narcissistic personality disorder is not included in ICD-10 or ICD-11, but is included in the DSM-5 Cluster B group. The diagnosis is rarely made outside North America. People with narcissistic personality disorder have a grandiose sense of self-importance. They may be preoccupied by fantasies of success, power, and brilliance, and believe that it is their right to receive special treatment. A high degree of egocentricity occurs in many of the other personality disorders, and so this trait is not in itself diagnostic. In antisocial personality disorder it is associated with a more malevolent feeling towards others, while those with narcissistic personalities are well-disposed, believing that other people admire them.

Ref: Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 201 p. 118.

The correct answer is: There may be a preoccupation with fantasies of success, power, and brilliance

Question **104**

Not answered

Marked out of 1.00

Flag question

The duration criteria set out in ICD-11 for diagnosing personality disorders is

Select one:

- 6 months
- 2 years
- 3 years
- 1 year
- None of the listed options

Explanation:

Personality disorder is characterised by problems in functioning of aspects of the self (e.g., identity, self-worth, accuracy of self-view, self-direction), and/or interpersonal dysfunction (e.g., ability to develop and maintain close and mutually satisfying relationships, ability to understand others' perspectives and to manage conflict in relationships) that have persisted over an extended period of time (e.g., 2 years or more). This time frame is used in the description but it is not used as a duration criterion in the description of individual syndromes.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: None of the listed options

Question **105**

Not answered

Marked out of 1.00

Flag

Which one among the following symptoms is not classified under Schneiderian first rank symptoms of schizophrenia?

Select one:

- Thought insertion

question

- Delusional perception
- Waxy flexibility
- Third person singular hallucinations
- Thought broadcasting

Explanation:

First rank symptoms of schizophrenia are useful because they occur reasonably often in schizophrenia and more rarely in other disorders, and it is not too difficult to tell whether they are present or not. First rank symptoms can be categorised as follows:

- Auditory hallucinations (voices heard arguing, thought echo, running commentary)
- Delusions of thought interference (thought insertion, thought withdrawal, thought broadcasting)
- Delusions of control (passivity of affect, passivity of impulse, passivity of volitions, somatic passivity)
- Delusional perception (a primary delusion of any content that is reported by the patient as having arisen, following the experience of a normal perception)

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 78.

The correct answer is: Waxy flexibility

Question **106**

Not answered

Marked out of 1.00

Flag question

Which of the following is true when comparing ICD-11 and DSM-5?

Select one:

- Only ICD-11 incorporates dimensional features into its diagnostic and classification system
- Only DSM-5 is a classic hierarchical categorical classification system
- Only ICD-11 is fundamentally descriptive in nature
- Both are based primarily on self-reported symptoms, clinically observed signs, and a few tests
- Neither ICD-11 nor DSM-5 allow for multiple comorbidities

Explanation:

Both ICD-11 and DSM-5 are fundamentally descriptive in nature and based primarily on self-reported symptoms, clinically observed signs, and a few tests (e.g., of intellectual functioning). A main characteristic of both ICD-11 and DSM-5 is that they are classic hierarchical categorical classification systems. Hierarchical categorical classification systems are exhaustive, their elements are mutually exclusive, and lower levels of the structure are related systematically to specific higher levels. Both ICD-11 and DSM-5 acknowledge the existence of dimensional features that are relevant to mental disorder and incorporate dimensional features into their diagnostic and classification systems. Both ICD-11 and DSM-5 permit multiple comorbidities (i.e., allow a person to meet the criteria for major depression, an anxiety disorder, and a psychotic disorder). Resulting diagnoses may misleadingly suggest that the individual has three disorders rather than a single, complex mental disorder.

Ref: Clark LA et al. Three approaches to understanding and classifying mental disorder: ICD-11, DSM-5, and the National Institute of Mental Health's Research Domain Criteria (RDoC). Psychological Science in the Public Interest. 2017 Nov;18(2):72-145.

The correct answer is: Both are based primarily on self-reported symptoms, clinically observed signs, and a few tests

Question **107**

Not answered

Marked out of 1.00

Flag question

Of those listed, the most common co-morbid psychiatric disorder seen in children with ADHD is

Select one:

- Conduct disorder
- Oppositional defiant disorder
- Bipolar disorder
- Substance abuse
- Sleep disorders

Explanation:

50-80% of children with ADHD have a comorbid disorder. **Common comorbid conditions** include specific learning disorders (70%), autism spectrum disorder (59%), tic disorders/Tourette syndrome (55%), oppositional defiant disorder (30-50%) depression (12-50%), bipolar disorder (5-47%), anxiety disorders (15-35%), conduct disorder (3.5-10%), sleep disorders, and substance misuse.

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

Gnanavel S et al. Attention deficit hyperactivity disorder and comorbidity: A review of literature. World Journal of Clinical Cases. 2019 Sep 6;7(17):2420.

The correct answer is: Oppositional defiant disorder

Question **108**

Not answered

Marked out of 1.00

Flag question

A middle-aged woman went with her husband to see her psychiatrist. She hasn't been sleeping or eating well for the last 4 weeks. She also complains that her bowels are dead and are no longer working. According to ICD-11 the severity of her depression can be classified as

Select one:

- Dysthymia
- Resistant
- Mild
- Moderate or severe
- She has no depression

Explanation:

In ICD-11, psychotic symptoms (delusions or hallucinations) can be present in either a moderate or severe depressive episode. In a moderate depressive episode, several depressive symptoms are present to a marked degree, or a large number of depressive symptoms of a lesser severity are present overall. An individual with a moderate depressive episode typically has considerable difficulty in continuing with work, social, or domestic activities, but is still able to function in at least some areas. In a severe depressive episode, many or most symptoms of a depressive episode are present to a marked degree, or a smaller number of symptoms are present and manifest to an intense degree and the individual is unable to function in personal, family, social, educational, occupational, or other important domains,

except to a very limited degree.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Moderate or severe

Question **109**

Not answered

Marked out of 1.00

Flag question

A 55-year-old woman with suspected multiple sclerosis presents with loss of function in one arm. She does not seem concerned by this defect. This is best described as

Select one:

- Echo de la pensee
- Moria
- Palimpsest
- Bouffee delirante
- La belle indifference

Explanation:

(La) belle indifference is a surprising lack of concern for, or denial of, apparently severe functional disability. It is part of classical descriptions of hysteria and continues to be associated with operational descriptions of conversion disorder. It is also seen in medical illnesses (e.g. following a stroke) and is a rare and non-specific symptom of no diagnostic value.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 103.

The correct answer is: La belle indifference

Question **110**

Not answered

Marked out of 1.00

Flag question

Which of the following traits in a woman with borderline personality disorder would be most likely to decrease at least partially over 2 years' time, with or without treatment?

Select one:

- Affective instability
- Anger control
- Emptiness
- Self-harm
- Unstable relationships

Explanation:

The course of core features of borderline personality disorder persists throughout adulthood, such as affective symptoms (chronic dysphoria, anger, and feelings of emptiness) and interpersonal symptoms related to fears of abandonment. Whereas impulsivity, self-injurious, and suicidal behaviour decrease during adulthood. Collaborative Longitudinal Personality Disorders Study (CLPS) by Gundersen et al. reported that self-harm improves longitudinally over a period of more than 2 years in those with borderline personality disorder.

Ref: Videler AC et al. A life span perspective on borderline personality disorder. Current psychiatry reports. 2019 Jul;21(7):1-8.

The correct answer is: Self-harm

Question **111**

Not answered

Marked out of 1.00

Flag question

A 21-year-old man moved from London to Dundee, a month ago. He is complaining of uneasiness, difficulty concentrating, and a feeling of inability to cope since the move. The diagnosis is

Select one:

- Generalized anxiety disorder
- Neurasthenia
- Social phobia
- Adjustment disorder
- Acute stress reaction

Explanation:

Adjustment disorder is a maladaptive reaction of an identifiable psychosocial stressor or multiple stressors that usually emerges within a month of the stressor. The disorder is characterised by preoccupation with the stressor or its consequences, including excessive worry, recurrent and distressing thoughts about the stressor, or constant rumination about its implications, as well as by failure to adapt to the stressor that causes significant impairment in personal, family, social, educational, occupational, or other important areas of functioning.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Adjustment disorder

Question **112**

Not answered

Marked out of 1.00

Flag question

A 33-year-old man has experienced low mood, anhedonia, fatigue, early morning wakening, and negative cognitions for the last 2 weeks. There is no personal or family history of mood disorders. He has been generally healthy apart from an episode of flu that occurred 2 weeks ago. Which of the following is an appropriate diagnosis?

Select one:

- Chronic fatigue syndrome
- Bipolar disorder
- Encephalitis
- No mental illness
- Depressive episode

Explanation:

A depressive episode is characterised by a period of depressed mood or diminished interest in activities occurring most of the day, nearly every day during a period lasting at least two weeks accompanied by other symptoms such as difficulty concentrating, feelings of worthlessness or excessive or inappropriate guilt, hopelessness, recurrent thoughts of death or suicide, changes in appetite or sleep,

psychomotor agitation or retardation, and reduced energy or fatigue.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Depressive episode

Question **113**

Not answered

Marked out of 1.00

Flag question

In ICD-10, schizotypal disorder was classified under

Select one:

- Disorders of psychological development
- Schizophrenia and related disorders
- Disorders of adult personality and behaviour
- Cluster A personality disorders
- Persistent delusional disorder

Explanation:

Schizotypal disorder is classified along with schizophrenia and other primary psychotic disorders in ICD-11 but along with Cluster A personality disorders in DSM-5. Schizotypy shares some of the clinical features of schizophrenia, but not the delusions or hallucinations. It is thought to be related to schizophrenia because schizotypy is more common in the other first-degree relatives of schizophrenic subjects than in the general population and the relatives for schizotypal subjects have an increased risk of schizophrenia.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Schizophrenia and related disorders

Question **114**

Not answered

Marked out of 1.00

Flag question

An 18-year-old woman has a BMI of 16. She exercises for a few hours every day and spends at least an hour eating each of her meals, slowly chewing very small amounts of food. She repeatedly weighs herself on a bathroom scale multiple times a day. What is the most appropriate diagnosis?

Select one:

- Anorexia nervosa with significantly low body weight, restricting pattern
- Anorexia nervosa with dangerously low body weight, restricting pattern
- Anorexia nervosa with normal body weight
- Anorexia nervosa with significantly low body weight, binge-purge pattern
- Anorexia nervosa with dangerously low body weight, binge-purge pattern

Explanation:

Anorexia nervosa is characterised by significantly low body weight for the individual's height, age, and developmental stage that is not due to another health condition or to the unavailability of food. 'Significantly low body weight' is considered to be a BMI between 18.5 and 14 for adults. 'Dangerously low body weight' is considered to be a BMI under 14. The 'restricting pattern' specifier should be assigned to individuals who induce weight loss and maintain low body weight through

restricting food intact or fasting alone or in combination with increased energy expenditure (e.g., through excessive exercise) but who do not engage in binge eating or purging behaviours.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Anorexia nervosa with significantly low body weight, restricting pattern

Question **115**

Not answered

Marked out of 1.00

Flag question

In which of the following personality disorders, the patient shows a preoccupation to avoid criticism and rejection?

Select one:

- Dependent
- Avoidant
- Borderline
- Passive aggressive
- Obsessive-compulsive

Explanation:

Clinical features of patients with avoidant personality disorder include avoiding jobs with significant interpersonal contact for fear of criticism or rejection; unwillingness to get involved unless certain of being liked; restraint in relationships because of fear of being ridiculed; preoccupation with being criticized or rejected; social inhibition because of feelings of inadequacy; viewing self as inferior; reluctance to take risks because of possible embarrassment.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 565.

The correct answer is: Avoidant

Question **116**

Not answered

Marked out of 1.00

Flag question

People with which of the following personality disorders find it hard to make decisions easily due to their rigidity in thinking patterns?

Select one:

- Schizotypal personality
- Narcissistic personality
- Schizoid personality
- Anxious avoidant personality
- Obsessive-compulsive personality

Explanation:

People with OCPD find it hard to decide on even seemingly trivial issues, leading to frustration at times.

Clinical features of patients with obsessive compulsive personality disorder include reluctance to delegate; perfectionism interfering with completion of tasks; miserly spending and hoarding of money; inability to discard worthless objects; excessive devotion to work and productivity; preoccupation with

details, rules, lists, order, schedules; inflexibility about morality and ethics; rigidity and stubbornness.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 566.

The correct answer is: Obsessive-compulsive personality

Question **117**

Not answered

Marked out of 1.00

Flag question

Mr.X is a 59-year-old gentleman who has a 6 month history of persecutory delusions about being spied on at home by his neighbours. There is no evidence of functional impairment. There is no history of past psychiatric illness or substance misuse. His most likely diagnosis is

Select one:

- Paranoid personality disorder
- Schizophrenia
- Schizotypal disorder
- Delusional disorder
- Schizoaffective disorder

Explanation:

Delusional disorder is characterised by the development of a delusion or set of related delusions, typically persisting for at least 3 months and often much longer, in the absence of a depressive, manic, or mixed mood episode. The delusions are variable in content across individuals, but typically stable within individuals, although they may evolve over time. Other characteristic symptoms of schizophrenia (i.e. clear and persistent hallucinations, negative symptoms, disorganised thinking, or experiences of influence, passivity, or control) are not present, although various forms of perceptual disturbances (e.g. hallucinations, illusions, misidentification of persons) thematically related to the delusion are still consistent with the diagnosis. Apart from the actions and attitudes directly related to the delusional system, affect, speech, and behaviour are typically unaffected.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Delusional disorder

Question **118**

Not answered

Marked out of 1.00

Flag question

A 13-year-old boy was sent from school by his teacher as he smelt of alcohol. The GP refers the family to child psychiatric services where his dad reports that his son is less interested in exploring new things. He never joins in on a conversation when there are visitors at home. He gets distressed and sweats profusely when attending parties and other occasions. Most likely diagnosis is

Select one:

- Specific phobia
- Alcohol dependence
- Agoraphobia
- Depression
- Social anxiety disorder

Explanation

Social anxiety disorder (social phobia) is characterised by marked and excessive fear or anxiety that consistently occurs in one or more social situations such as social interactions, doing something while feeling observed, or performing in front of others. The individual is concerned that they will act in a way, or show anxiety symptoms, that will be negatively evaluated by others. Relevant social situations are consistently avoided or else endure with intense fear or anxiety. Somatic symptoms include blushing, trembling, dry mouth, and perspiration. There is a high level of comorbidity with substance misuse.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 378.

The correct answer is: Social anxiety disorder

Question **119**

Not answered

Marked out of 1.00

Flag question

A 26-year-old man was diagnosed with schizophrenia 5 years ago. He has experienced significant persecutory delusions and auditory hallucinations for most of that time, despite being prescribed antipsychotic medication. However, he has not necessarily taken any medication consistently. There have been brief periods of time when the voices have receded, but he reports that 'they always find me again.' What is his most appropriate current diagnosis?

Select one:

- Schizophrenia, first episode
- Schizophrenia, multiple episodes
- Schizophrenia, continuous
- Schizophrenia, episode unspecified
- Other specified episode of schizophrenia

Explanation:

'Schizophrenia, continuous' should be diagnosed if symptoms fulfilling schizophrenia have been present for almost all of the illness course over a period of at least one year, with periods of subthreshold symptoms being very brief relative to the overall course. Note that if the duration of the first episode of schizophrenia is more than 1 year, the 'continuous' specifier should be used instead.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Schizophrenia, continuous

Question **120**

Not answered

Marked out of 1.00

Flag question

How long does the natural course of an episode of untreated mania last?

Select one:

- 4 months
- 9 months
- 6 weeks
- 2 weeks
- 4 weeks

Except in the elderly, the natural course of mania lasts for 4 months while depression for 6 months. This becomes longer in the elderly who show shorter periods of interepisodic remissions and more frequent episodes which are considerably longer.

The correct answer is: 4 months

Question **121**

Not answered

Marked out of 1.00

Flag question

A 21-year-old man has been elated for the last 2 weeks with reduced need for sleep. On mental state examination, he has pressured speech, racing thoughts and grandiose delusions. The diagnosis is

Select one:

- Acute stress reaction
- Hypomania with psychosis
- Mania with psychosis
- Hypomania without psychosis
- Mania without psychosis

Explanation:

Clinical features of mania:

- Elevated mood (out of keeping with circumstances)
- Increased energy which may manifest as overactivity and/or reduced need for sleep
- Formal thought disorder which may manifest as pressured speech, flight of ideas, and/or racing thoughts
- Increased self-esteem, evident as over-optimistic ideation, grandiosity, reduced social inhibitions, over-familiarity
- Reduced attention and increased distractibility
- Tendency to engage in behaviour that may have serious consequences (e.g. spending recklessly, inappropriate sexual encounters)
- Other behavioural manifestations, including excitement, irritability, aggressiveness, and suspiciousness
- Marked disruption of work, usual social activities, and family life

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 320-321.

The correct answer is: Mania with psychosis

Question **122**

Not answered

Marked out of 1.00

Flag question

A 40-year-old man with poor medication compliance has a long-running history of dysthymia with episodes of hypomania on prescribing antidepressants. Which of the following might best describe his diagnosis?

Select one:

- bipolar II disorder
- double depression
- cyclothymia

- bipolar I disorder
- bipolar III disorder

Explanation:

"Bipolar III" is characterised by hypomania or mania precipitated by antidepressant drugs. This appears in DSM-5 as 'Substance/medication-induced bipolar and related disorder' but is not called 'BipolarIII disorder'.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 324.

The correct answer is: bipolar III disorder

Question **123**

Not answered

Marked out of 1.00

Flag question

Which of the following is associated with an increased risk of developing borderline personality disorder?

Select one:

- Alcohol abuse
- Sexual abuse
- Bullying at school
- Drug abuse
- Death of mother before the age of 12

Explanation:

High rates of physical, sexual, and emotional abuse are reported in people with borderline personality disorder.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of borderline personality disorder. Updated edition 2018.

The correct answer is: Sexual abuse

Question **124**

Not answered

Marked out of 1.00

Flag question

One of your patients has a co-morbid paranoid personality disorder with a delusional disorder. Identify the correct statement regarding delusional disorder.

Select one:

- Depressive episodes invalidate the diagnosis
- Delusional disorders are associated with blunted affect
- The delusions must be present for a period of at least 3 months
- Passivity phenomenon is an important feature
- Persistent hallucinations are notable in most but not all patients

Explanation:

Delusional disorder is characterised by the development of a delusion or set of related delusions, typically persisting for at least 3 months and often much longer, in the absence of a depressive, manic, or mixed mood episode. The delusions are variable in content across individuals, but typically stable within individuals, although they may evolve over time. Other characteristic symptoms of schizophrenia (i.e. clear and persistent hallucinations, negative symptoms, disorganised thinking, or experiences of influence, passivity, or control) are not present, although various forms of perceptual disturbances (e.g. hallucinations, illusions, misidentification of persons) thematically related to the delusion are still consistent with the diagnosis.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: The delusions must be present for a period of at least 3 months

Question **125**

Not answered

Marked out of 1.00

Flag question

Mrs. Q is a 43-year-old woman who has a long standing history of mood disorder for over 20 years. Over the last year alone she has suffered from 5 mood episodes and is currently on treatment with semisodium valproate. Which one among the following is particularly associated with her condition?

Select one:

- Hyperthyroidism
- Antidepressant use
- Alcohol Abuse
- Menopause
- Stressful life events

Explanation:

There is an association between bipolar rapid cycling (four or more episodes per year) and antidepressant use. Since episodes of depression (which then trigger the prescription of an antidepressant) often precede rapid cycling, the relationship may be associative rather than causal.

Ref: Gitlin MJ. Antidepressants in bipolar depression: an enduring controversy. International journal of bipolar disorders. 2018 Dec;6(1):1-7.

The correct answer is: Antidepressant use

Question **126**

Not answered

Marked out of 1.00

Flag question

According to ICD-11 criteria, what is the minimum duration required to make a diagnosis of alcohol dependence if use is almost daily?

Select one:

- 2 months
- 3 months
- 1 month
- 12 months
- 6 months

Explanation:

In ICD-11, features of alcohol dependence are usually evident over a period of at least 12 months but the diagnosis may be made if alcohol use is continuous (daily or almost daily) for at least 1 month.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: 1 month

Question **127**

Not answered

Marked out of 1.00

Flag question

Which of the following features is commonly seen in borderline personality disorder?

Select one:

- Interpersonal anxiety
- Lack of empathy
- Chronic feelings of emptiness
- Marked tendency to quarrelsome behaviour
- Marked tendency to act unexpectedly and without consideration of the consequences

Explanation:

Clinical features of patients with borderline personality disorder include: unstable self-image; unstable interpersonal relationships; impulsive behaviour in areas such as personal safety and substance misuse; self-harm, suicidal threats or gestures; affective instability due to marked reactivity of mood; episodes of micro-psychosis; chronic feelings of emptiness; frantic efforts to avoid real or imagined abandonment; inappropriate, intense anger.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 564.

The correct answer is: Chronic feelings of emptiness

Question **128**

Not answered

Marked out of 1.00

Flag question

Miss X is a 26-year-old woman who has been treated recently for hypomania with a mood stabiliser. She also has a past history of depression 3 years ago. Which one among the following is her diagnosis according to ICD-11 criteria?

Select one:

- Recurrent depressive disorder
- Mixed affective state
- Rapid cycling disorder
- Bipolar disorder type 2
- Bipolar disorder type 1

Explanation:

Diagnosis of bipolar disorder:

- In **DSM-5** and **ICD-11** bipolar disorders are **subdivided** into **bipolar I** and **bipolar II** disorder
- **Bipolar I** requires **at least one past or present manic or mixed episode**; **depressive episodes** may occur but **are not obligatory**

- **Bipolar II** requires **at least one hypomanic and one depressive episode**

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 328.

The correct answer is: Bipolar disorder type 2

Question **129**

Not answered

Marked out of 1.00

Flag question

Which one of the following was a key proponent of the view that mental diseases are diseases of the brain?

Select one:

- Wilhelm Griesinger
- Adolf Meyer
- Emil Kraepelin
- Karl Jaspers
- Charcot

Explanation:

Wilhelm Griesinger (1817-1868) is often referred to as the 'founding father of neuropsychiatry' (perhaps more accurately the first 'biological psychiatrist'). It is to him that the (apocryphal) quotation has been attributed that all mental diseases are just diseases of the brain. His Textbook of Mental Pathology and Therapeutics (1845) was hugely influential.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 122.

The correct answer is: Wilhelm Griesinger

Question **130**

Not answered

Marked out of 1.00

Flag question

The most vocationally disabling personality disorder is

Select one:

- Schizoid
- Borderline
- Antisocial
- Paranoid
- Obsessive-compulsive

People with either borderline or schizotypal personality disorder have significantly more occupational impairment than patients with obsessive-compulsive personality disorder. (Skodol 2002)

The correct answer is: Borderline

Question **131**

Not answered

Which one of the following statements about Rett syndrome is true?

Marked out of 1.00

Flag question

Select one:

- There is apparent lack of normal development in the first 2 years of life
- Is associated with microcephaly at birth
- Occurs mainly in boys
- Characterised by progressive loss of acquired abilities
- Is associated with borderline IQ

Explanation:

Rett syndrome:

· Believed to be caused by **dominant X-linked gene**

· **Degenerative** disorder that almost exclusively affects **females**

· Initially normal development

· Ages 1- 4 yrs: purposeful hand movements & spoken language lost; stereotypical hand movements (**hand-wringing**) & autism seen

· Ataxia, facial grimacing, teeth-grinding, and loss of speech

· **Irregular respiration** with episodes of hyperventilation, apnea, and breath-holding

· Progressive **gait disturbance, scoliosis,** and **seizures**

· **Sleep disturbances** are common; patients suffer from initial and/or intermediate insomnia, leading to excessive daytime sleep

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 103.

Persico AM et al. The psychopharmacology of autism spectrum disorder and Rett syndrome. In Handbook of clinical neurology 2019 Jan 1 (Vol. 165, pp. 391-414). Elsevier.

The correct answer is: Characterised by progressive loss of acquired abilities

Question **132**

Not answered

Marked out of 1.00

Flag question

Classificatory systems can be used to make a diagnosis based on inclusion and exclusion criteria. Which of the following properties of classificatory systems aid in the above?

Select one:

- Operationalised approach
- Atheoretical approach
- Multiaxial approach
- Aetiological approach
- Descriptive approach

Explanation:

An operationalised approach involves developing clearly defined clinical descriptions of disorders, together with explicit inclusion and exclusion criteria and details of the number and duration of symptoms required for diagnosis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 10.

The correct answer is: Operationalised approach

Question **133**

Not answered

Marked out of 1.00

Flag question

A 23-year-old gentleman was started on an anti-psychotic medication 3 weeks ago. During a follow-up appointment he reports no improvement in his symptoms. What is the most sensible next step?

Select one:

- Add another antipsychotic medication
- Augmentation with an anticonvulsant medication
- Explore potential non-adherence
- Increase the dose of the antipsychotic medication
- Change to another antipsychotic drug

Explanation:

Among people with schizophrenia, non-adherence with antipsychotic treatment is high. A great deal of poor adherence occurs without the knowledge of the prescriber. NICE recommends that the patient should be asked in a non-judgemental way if they have missed any doses over a specific time period such as the previous week.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 27, 898, 901.

The correct answer is: Explore potential non-adherence

Question **134**

Not answered

Marked out of 1.00

Flag question

Which one of the following eponymous delusional disorders are not correctly matched?

Select one:

- Capgras syndrome - l'illusion des sosies
- Othello syndrome - morbid jealousy
- Fregoli syndrome - delusional dysmorphophobia
- Ekbom syndrome - delusion of infestation
- De Clerambault syndrome - erotomania

Explanation:

Capgras syndrome (l'illusion des sosies): a type of delusional misidentification in which the patient believes that a person known to them has been replaced by a 'double' who is to all external appearances identical but is not the 'real person'.

Othello syndrome: a monosymptomatic delusional disorder where the core delusion has the content of delusional jealousy. Patients possess the fixed belief that their spouse or partner has been unfaithful. Often patients try to collect evidence and/or attempt to restrict their partner's activities. Can be associated with forensic cases involving murder.

Fregoli syndrome: a type of delusional misidentification, in which the patient believes that strangers have been replaced with familiar people.

Ekbom syndrome: a monosymptomatic delusional disorder where the core delusion is a delusion of infestation.

De Clerambault syndrome: a form of delusion of love in which the patient, usually female, believes

that another, higher-status individual is in love with them.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 104-105, 109, 111, 115, 230, 240.

The correct answer is: Fregoli syndrome - delusional dysmorphophobia

Question **135**

Not answered

Marked out of 1.00

Flag question

Which of the following traits is seen in paranoid personality disorder?

Select one:

- Fear of abandonment and relying on other people
- Inability to make decisions
- Increased sensitivity to criticism and setbacks
- Does not learn from mistakes
- Strives for perfection

Explanation:

Patients with paranoid traits are often combative and highly suspicious in nature; this may elicit a hostile response in others. They are often critical of others, but have great difficulty in handling even a minor criticism from others.

Characteristics of patients with paranoid personality disorder include:

- Suspicions that others are exploiting, harming, or deceiving them
- Reading hidden demeaning or threatening meanings into benign remarks/events
- Holding long-standing grudges
- Believing that others are not loyal or trustworthy
- Reluctance to confide in others
- Feeling that others are attacking their character or reputation
- Having recurrent suspicions of infidelity

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 562.

The correct answer is: Increased sensitivity to criticism and setbacks

Question **136**

Not answered

Marked out of 1.00

Flag question

A 27-year-old woman with no previous medical or psychiatric history presents with a two-week history of insomnia, grandiosity, and increased energy. She is in a highly distractible state with a somewhat elated mood. You attempt to do an MMSE, and she scores 23/30. Which is the most likely diagnosis?

Select one:

- Herpes simplex encephalitis
- Delirium on dementia
- Bipolar affective disorder
- Evolving delirium
- Acute stress reaction

Explanation:

Clinical features of mania:

- Elevated mood (out of keeping with circumstances)
- Increased energy which may manifest as overactivity and/or reduced need for sleep
- Formal thought disorder which may manifest as pressured speech, flight of ideas, and/or racing thoughts
- Increased self-esteem, evident as over-optimistic ideation, grandiosity, reduced social inhibitions, over-familiarity, and/or facetiousness
- Reduced attention and increased distractibility
- Tendency to engage in behaviour that may have serious consequences (e.g. preoccupation with extravagant, impractical schemes, spending recklessly, inappropriate sexual encounters)
- Other behavioural manifestation, including excitement, irritability, aggressiveness, and suspiciousness
- Marked disruption of work, usual social activities, and family life

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 320-321.

The correct answer is: Bipolar affective disorder

Question **137**

Not answered

Marked out of 1.00

Flag question

A 30-year-old man is involved in a motorcycle accident, in which his girlfriend (passenger) dies. He presents with paralysis of his left arm, stating he cannot ride a motorcycle again. The doctors find no organic cause. What is the most appropriate diagnosis?

Select one:

- Dissociative neurological symptom disorder
- Post-traumatic stress disorder
- Adjustment disorder
- Hypochondriasis
- Neurasthenia

Explanation:

Dissociative neurological symptom disorder, with paresis or weakness is characterised by a difficulty or inability to intentionally move parts of the body or to coordinate movements that is not consistent with a recognised disease of the nervous system, other mental, behavioural or neurodevelopmental disorder, or other medical condition.

Dissociative disorders are presumed to be "psychogenic" in origin, being associated closely in time with traumatic events, insoluble and intolerable problems, or disturbed relationships. The onset and termination of dissociative states are often reported as being sudden, and the symptoms usually develop in close relationship to psychological stress. The commonest varieties are loss of ability to move the whole of part of a limb or limbs. Paralysis may be partial, with movements being weak or slow, or complete. Various forms and variable degrees of incoordination (ataxia) may be evident, particularly in the legs, resulting in bizarre gait or inability to stand unaided (astasia-abasia).

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Dissociative neurological symptom disorder

Question **138**

Not answered

Marked out of 1.00

Flag question

A patient with schizophrenia becomes socially withdrawn after experiencing delusions of persecution. This social withdrawal is most accurately classified as a

Select one:

- Primary negative symptom
- Secondary negative symptom
- Positive symptom
- Catatonic symptom
- Mood symptom

Explanation:

Primary negative symptoms are intrinsic to the underlying pathophysiology of schizophrenia, while secondary negative symptoms are related to other factors. Secondary negative symptoms, which can respond to treatment, occur in association with or result from positive symptoms, affective symptoms, medication side effects, environmental deprivation, or other treatment- or illness-related factors. For example, negative symptoms can be a secondary effect of primary positive symptoms in a patient who becomes socially withdrawn after experiencing delusions of persecution or paranoia, as is the case here.

Ref: Correll CU, Schooler NR. Negative symptoms in schizophrenia: A review and clinical guide for recognition, assessment, and treatment. Neuropsychiatric disease and treatment. 2020 Feb 21.

The correct answer is: Secondary negative symptom

Question **139**

Not answered

Marked out of 1.00

Flag question

A 40-year-old woman had an episode of depression 4 years ago. For last 4 weeks she is low in mood, socially isolating herself, and fears going out of her home. She fears experiencing a panic attack if she goes out. She is quite tearful and has not slept much for the last four weeks. The most appropriate working diagnosis is

Select one:

- Agoraphobia
- Depressive relapse
- Generalised anxiety
- Dysthymia
- Panic disorder

Explanation:

Depressive symptoms include low mood, anhedonia, weight change, disturbed sleep, psychomotor agitation or retardation, fatigue or loss of energy, reduced libido, feelings of worthlessness or excessive or inappropriate guilt, diminished ability to think or concentrate or indecisiveness, recurrent thoughts of death or suicide.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p.

246.

The correct answer is: Depressive relapse

Question **140**

Not answered

Marked out of 1.00

Flag question

A 55-year old man has been suffering from short-term memory problems, and other problems related to concentration. Otherwise, his daily activities are not affected. Which of the following is he most likely to develop?

Select one:

- Vascular dementia
- Lewy body dementia
- Huntington chorea
- Creutzfeldt Jacob disease
- Alzheimer dementia

Explanation:

Mild cognitive impairment (MCI), particularly in its amnestic form, is often a precursor to Alzheimer disease (AD). Approximately 10-15% of people with amnestic MCI progress to AD each year.

Ref: Anderson ND. State of the science on mild cognitive impairment (MCI). CNS spectrums. 2019 Feb;24(1):78-87.

The correct answer is: Alzheimer dementia

Question **141**

Not answered

Marked out of 1.00

Flag question

The term that refers to the neglect of all leisure and other alternate forms of pleasure apart from alcohol is

Select one:

- Dependence
- Tolerance
- Narrowing of repertoire
- Reinstatement
- Salience

Explanation:

Salience/primacy of alcohol-seeking behaviour:

- Refers to the **neglect of all leisure and alternate forms of pleasure** apart from alcohol
- **Alcohol becomes more important** than retaining a job or relationships or remaining financially solvent and in good physical health
- May **diminish the moral sense**, leading to criminal activity and fraud

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British

medical journal. 1976 May 1;1(6017):1058.

The correct answer is: Salience

Question **142**

Not answered

Marked out of 1.00

Flag question

A 32-year-old woman was brought by the Police to the A and E Department after she was found attempting to gain access to a hotel room where a popular football player was staying. She stated that the football player invited her into his hotel room and also claimed that they were secretly married. The football player denied this but admitted that the woman had sent him hundreds of emails over the last year. The woman had not been in trouble with the law so far and was functioning well in the community. Which of the following is the most likely diagnosis?

Select one:

- De Clerambault syndrome
- Othello syndrome
- Fregoli syndrome
- Ekbom syndrome
- Capgras syndrome

Explanation:

De Clerambault syndrome is a form of delusion of love. The patient, usually female, believes that another, higher status individual is in love with them and may make efforts to contact that person. There may be an additional persecutory delusional component where the affected individual comes to believe that individuals are conspiring to keep them apart.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 105.

The correct answer is: De Clerambault syndrome

Question **143**

Not answered

Marked out of 1.00

Flag question

For 3 weeks after an automobile collision where her husband died in front of her, a schoolteacher suffers from insomnia, poor concentration and tearfulness. She has nightmares and emotional outbursts when awake. Which of the following is the most likely diagnosis?

Select one:

- post-traumatic stress disorder
- acute stress disorder
- dysthymia
- adjustment disorder
- depressive episode

Explanation:

Acute stress disorder is similar to PTSD with symptoms in the categories of re-experiencing of events (intrusion), avoidance, negative mood, and hyperarousal, but lasting no more than 4 weeks.

Such acute responses are higher in t he presence of physical exhaustion, organic factors or disease states. The stressor is usually one that poses a serious threat to security, integrity and social position. The patient may initially be dazed with narrowed attention; disorientation is not uncommon as a result. Sometimes agitation and overactivity are seen. Partial or complete amnesia is not unheard of. Dissociative symptoms seem to predominate in some.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 394.

The correct answer is: acute stress disorder

Question **144**

Not answered

Marked out of 1.00

Flag question

Which one of the following is an ICD-11 diagnostic feature of frontotemporal dementia?

Select one:

- Insidious onset and gradual progression
- 3-6 month history of cognitive decline
- Memory impairment precedes behavioural manifestations
- Older age of onset
- Abrupt onset and rapid progression

Explanation:

ICD-11 characterises frontotemporal dementia (FTD) as a group of primary neurodegenerative disorder primarily affecting the frontal and temporal lobes. Onset is typically insidious with a gradual and worsening course. Several syndromic variants are described that include presentations with predominantly marked personality and behavioural changes or with predominantly language deficits, or with a combination of these deficits. Memory function, psychomotor speed, as well as visuoperceptual and visuospatial abilities often remain relatively intact, particularly during the early stages of the disorder.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: Insidious onset and gradual progression

Question **145**

Not answered

Marked out of 1.00

Flag question

A school teacher suspects autism spectrum disorder in a young boy. Which of the following would support her concerns?

Select one:

- Poor learning of grammar
- Other teachers report that he has many friends
- He wants to be a gardener
- The mum reports that he did not speak at all before the age of 3
- Monotonous voice and no eye contact

Explanation:

Autism spectrum disorders are a group of lifelong developmental disorders characterised by their effect on social and communication skills, as well as by a restricted, stereotyped, repetitive repertoire of interests and activities. Clinical features include: few or no sustained relationships; odd voice (monotonous and perhaps at an unusual volume, talking at rather than to you); good superficial language but too formal, stilted, or pedantic; impassive appearance with few gestures and abnormal gaze (i.e. limited non-verbal communicative behaviour); focus on rules.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 674.

The correct answer is: Monotonous voice and no eye contact

Question **146**

Not answered

Marked out of 1.00

Flag question

Which of the following is indicative of disorganised thinking in schizophrenia?

Select one:

- Tangentiality
- Loose associations
- Neologisms
- Word salad
- All of the listed options

Explanation:

Disorganised thinking (formal thought disorder) is a feature of schizophrenia. This may include tangentiality and loose associations, irrelevant speech, and neologisms. When severe, the person's speech may be so incoherent as to be incomprehensible ('word salad').

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed, ICD-11; World Health Organization, 2020)

The correct answer is: All of the listed options

Finish review

PREVIOUS ACTIVITY
EMIs Development

NEXT ACTIVITY
Clinical Examination

Jump to...

Drop here!

© 2022 SPMM Course Limited, UK.

Refund Policy | Terms & Conditions

